

Patient Face Sheet

TRANSCEND HOSPICE 18 LLC

PATIENT INFORMATION	
Patient Name: SANDRA R HAMLIN	MR #: 90008
Admission Date: 11/4/2025	Admission #: 2 Current Status: Admitted (Hospice)
Primary Diagnosis (ICD10): I63.9 (Cerebral infarction, unspecified) Related Conditions (ICD10): R52 (Pain, unspecified), R11.2 (Nausea with vomiting, unspecified), K21.00 (Gastro-esophageal reflux dis with esophagitis, without bleed), E46 (Unspecified protein-calorie malnutrition), I21.9 (Acute myocardial infarction, unspecified), I50.22 (Chronic systolic (congestive) heart failure), I10 (Essential (primary) hypertension), E78.00 (Pure hypercholesterolemia, unspecified), J45.901 (Unspecified asthma with (acute) exacerbation), I25.10 (Atherosclerotic heart disease of native coronary artery w/o ang pcts), G40.901 (Epilepsy, unsp, not intractable, with status epilepticus), D26.9 (Other benign neoplasm of uterus, unspecified), R19.7 (Diarrhea, unspecified), R60.9 (Edema, unspecified) Unrelated Conditions (ICD10):	
Level of Care: Routine Home Care	Disaster Code: Disaster Code 2
General Information:	
Birth Date: 9/26/1943	SSN: 458-72-8838
Race: White	Gender: Female
Ethnicity: Caucasian	
Marital Status: Widowed	Is DNR?: Yes
Other Information:	Primary Language: English
Allergies:	Religion: Christian
Funeral Home:	
Address:	
Patient Contact Information:	
Referral Information:	
Referral Source:	Called In By: Laura Farrar
LOCATION INFORMATION	
Current Location:	
Location Type: Home	
Institution Name (facility patients):	
Address: 5813 Excalibur Dr, Garland TX 75044-3633	
CAREGIVER INFORMATION	
Primary Caregiver	
Caregiver Name: Susan Phillips	Relationship to Patient: Daughter
Address: 3409 Sugarberry Ln, Garland TX 75044-3609	
Mobile: (972) 849-7060	Email: homesuzan@yahoo.com
PHYSICIAN INFORMATION	
Attending Physician Name:	NPI #:
Address:	
Hospice Physician Name: RESTREPO, DIEGO	NPI #: 1063664076
FINANCIAL INFORMATION	
Payer Name: Palmetto GBA	
Coverage ID: 7TT4UP3MP93	Coverage Type: Medicare Part A
Service(s): Hospice (Primary)	

Medication List

HAMLIN, SANDRA (MR# 90008)

Generated On: 12/28/2025 8:42 PM

Allergies: <Blank>

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
12/3/2025		Furosemide	20	1	By mouth	Twice a day	Edema	Yes	Active
12/3/2025		Melatonin	10	1	By mouth	Qhs	Insomnia	Yes	Active
12/3/2025		Potassium	10	1	Po	Daily	Supplement	Yes	Active
12/3/2025		Quetiapine	50	1	Po	At bedtime	Mood disorder	Yes	Active
12/3/2025	12/3/2025	Meatonin	10	1	Po	Bedtime	Edema	Yes	Discontinued
Comfort Kit Medications:									
11/4/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/4/2025		Atropine sulfate oral Soluntion	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
11/4/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active
11/4/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/4/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/4/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/4/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/4/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Patient: SANDRA HAMLIN
Medical Record: #90008

INITIAL COMPREHENSIVE ASSESSMENT (WITH HOPE)

COMPREHENSIVE ASSESSMENT

Date/Time In: 11/4/2025 9:30:00 AM
Date/Time Out: 11/4/2025 10:45:00 AM
Modality: In-person visit

Describe onset of illness, progression of symptoms, functional status, response to treatments, etc: including any recent hospitalizations, ER visits, weight loss and infections: Sandra Hamlin, an 82 -year-old widowed female with a primary diagnosis of cerebral infarction, unspecified (I63.9), is currently under routine hospice services. She presents with related conditions including unspecified pain (R52), nausea with vomiting (R11.2), gastro-esophageal reflux disease with esophagitis, without bleeding (K21.00), and unspecified protein-calorie malnutrition (E46). Ms. Hamlin's current healthcare needs are being managed under the supervision of Hospice Physician Dr. Diego Restrepo.

Ms. Hamlin has a history of multiple hospital admissions for suspected cancer, CVA, MI, syncope, history of GT placement. She has right-sided weakness, severe hypertension, and an episode of syncope. Despite being sent home with private in-home care, Ms. Hamlin's condition has continued to decline, evidenced by a progressive weight loss of 11 pounds over the last two months, bringing her weight to 130 pounds and a BMI of 21. Her serum albumin level is notably low at 2.5 g/dL, indicating impaired nutritional status. Her Karnofsky Performance Status is 50%, and the Palliative Performance Scale (PPS) is also 50%, signifying moderate disease progression with significant care needs.

Physical assessment denotes the following:

- Height: 5'4"
- Mid-arm circumference (LMAC): 22 cm
- Activities of Daily Living (ADLs): 6/6, requiring total care in all areas.

Ms. Hamlin reports a general increase in lethargy, sleeping approximately 14 hours daily. Her current pain level, assessed using the Pain Assessment in Advanced Dementia (PAINAD) scale, is 2, suggesting mild pain.

Ms. Hamlin's care plan focuses on symptom management and comfort. Interventions include regular monitoring of her nutritional intake and hydration status, management of pain and other related symptoms, and providing ongoing emotional and spiritual support in accordance with her Christian faith. Discussions with her caregiver support the ongoing need for guidance and resources to manage her evolving care needs effectively. As per her wishes, she holds a Do Not Resuscitate (DNR) order. The patient is no longer seeking aggressive treatment.

CO-MORBIDITIES

What other diagnoses does patient have that may contribute to a decreased life expectancy when combined with the primary diagnosis?

- ☒ Congestive Heart Failure
- ☒ Ischemic Heart Disease
- ☒ Generalized Muscle Weakness
- ☒ Malnutrition
- ☒ COPD
- ☒ CKD
- ☒ Dementia

GENERAL INFORMATION

Race: Caucasian
Religion: Christian
Primary Language: English
Does the patient or primary caregiver need an interpreter? No
Is the patient or the patients spouse a veteran? No

PSYCHOSOCIAL / SPIRITUAL / BEREAVEMENT ASSESSMENT

Patient/Responsible Party's Current Response to Diagnosis/Prognosis: (Check all that apply)

• Acceptance

- ☒ Patient
- ☒ Representative

• Support Systems

- ☒ Church
- ☒ Neighbors
- ☒ Friends

• Spiritual Support

Needs spiritual support? Yes

• Psychosocial Support

Needs psychosocial support? Yes

• Patient/Family Goals

- ☒ Patient and family will receive physical, spiritual, and emotional support
- ☒ Effective grieving
- ☒ Safe and comfortable dying

PAIN/COMFORT ASSESSMENT

Has patient experienced pain the past 7 days? Yes

Pain Assessment Scale utilized: Verbal (scale of 1-10)

Acceptable level of pain: 2

Best: 0

Worst: 4

☒ Generalized pain/undefined location

Pain Frequency: Intermittent

Pain Duration: 1-4 hours

Bone/Somatic pain: Aching Dull

Visceral pain: Aching

Neuropathic pain: Burning

Non-Verbal Signs of Pain

- ☒ Moaning/crying
- ☒ Grimacing

What makes the pain better?

- ☒ Rest
- ☒ Breathing exercises
- ☒ Position change
- ☒ Medication
- ☒ Massage

What causes pain or makes it worse?

☒ Not taking pain medication

What other symptoms are associated with the pain?

☒ Decreased appetite

☒ Nausea/vomiting

Current treatments for pain:

☒ Opioids

Patient is on a scheduled opioid, currently taking a PRN opioid or has been instructed to begin taking an opioid: Yes

Specify the scheduled opioids the patient is currently taking (leave blank if n/a): Morphine

Specify the PRN opioids the patient is taking (leave blank if n/a): Morphine

Patient is on a bowel regime: Yes

Date bowel regime initiated: 11/6/2025

Describe bowel regime: Senna

How much breakthrough pain medication has patient taken in the past 24 hours? 1

Is current pain regime effective? Yes

Patient/family pain management goals Relieve pain/discomfort as much as possible even if it will cause increased sedation

Patient/caregiver educated on pain regime and side effects of pain medications: Yes, and verbalized understanding

Patient educated on non-medication treatments of pain? Yes

☒ Heat

☒ Cold

☒ Breathing exercises

Non-medication treatments were initiated: Yes

☒ Heat

☒ Cold

☒ Breathing exercises

☒ Massage

HEAD TO TOE ASSESSMENT

Vital Signs

Type of temperature taken: Temporal:

Temporal temperature: 97.6

Respiratory rate (respirations per minute): 16

Respiratory pattern: Regular

Pulse Type: Apical

Heart rate: 86

Blood pressure type: Sitting

Systolic: 157

Diastolic: 66

Height: 63

Weight: 122

Arm circumference taken: Left

Arm circumference: 20

Nutrition

☒ Diet Orders (specify):

☒ General (as tolerated)

Average percentage of meals consumed: 25

Patient Feeding: Needs cuing

Neurological Status

Orientation

☒ Self

Behavior

☒ Calm

☒ Withdrawn

☒ Confused

☒ Forgetful

Energy

☒ Fatigue/Weakness

☒ Increased sleep

Number of hours spent sleeping in a day: 13

Responsiveness

Responsiveness: Verbal stimuli

Intellect

Intellect: Confusion

Speech

☒ Clear

Neuromusculoskeletal Status

☒ Shuffling gait/uncoordinated gait

Muscle Tone

☒ Muscle wasting

Functional Assessment

Functional Assessment Score: 6E

Gastrointestinal Status

Last BM Today

Bowel Sounds positive in all 4 Quadrants

☒ Hypoactive

☒ RUQ

☒ RLQ

☒ LUQ

☒ LLQ

☒ Incontinence

Genitourinary Status

☒ Urinary incontinence

Integumentary Status

☒ Skin intact

Pressure Ulcer Risk Assessment

Sensory perception: ability to respond meaningfully to pressure-related discomfort:	1
Moisture: degree to which skin is exposed to moisture:	1
Activity: degree of physical activity:	1
Mobility: ability to change and control body position:	1
Nutrition: usual food intake pattern:	1
Friction and shear:	1
Total score:	6

CARDIOPULMONARY ASSESSMENT

Was patient screened for shortness of breath	1. Yes
Date of first screening for shortness of breath	11/4/2025
Did screening indicate the patient had shortness of breath	1. Yes
Was treatment for shortness of breath initiated	2. Yes
Date treatment for shortness of breath initiated	11/4/2025

Respiratory Treatments for Shortness of Breath:

☒ Oxygen saturation less than 88% on room air

☒ Oxygen

Liters: 2
Via: NC
Frequency: As needed

☒ Dyspnea

☒ At rest

☒ Cough

Frequency: Chronic
Type: Dry

☒ Capillary Refill (Fingers)

☒ More than 3 seconds

☒ Edema

Location: Lower legs
Type: Non-pitting

NY Heart Association Scale: Class IV (Severe)

FUNCTIONAL STATUS

Walking/Locomotion Walker
Bed Mobility; Assist of 1
Transfers: Assist of 1
Dressing: Independent
Eating: Independent
Toilet use: Needs Assistance
Personal hygiene: Dependent
Bathing Dependent
Palliative performance scale (PPS): 50% - Unable to do any Work

Fall Risk Assessment (NHPCO Guidelines)

• History of a fall within the last 6 months

☒ 2: 1 or 2 falls

• Mobility

☒ 3: Confined to bed or chair

• Elimination

☒ 2: Intermittent incontinence

• Mental Status

☒ 2: Intermittent confusion

• Use of medication

☒ 1: Narcotic

• Medical conditions which could impair the ability to ambulate

☒ 1: Weakness or paralysis of the extremities

Total: 11

☒ 10 or more points = High risk

HOME SAFETY ASSESSMENT

- ☒ No safety problems identified
☒ Emergency Preparedness plan given to patient

ASSESSMENT OF PATIENT/CAREGIVER TO SAFELY ADMINISTER MEDICATIONS

- ☒ Medication may be safely administered by:
☒ Caregiver
☒ Nurse
☒ Patient/ Caregiver/POA educated on medication regime and side effects
☒ Patient/Caregiver/POA verbalized understanding of Medication teaching provided

HOPE Admission Assessment

Based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?

0. No

Living Arrangements

2. With others in the home (family, friends, paid caregiver)

Availability of Assistance

1. Around-the-clock (24 hours a day with few exceptions)

Interpreter requested

0. No

Preferences

Was patient/responsible party asked about CPR 1. Yes

Date asked about CPR 11/4/2025

Was patient/responsible party asked about treatments other than CPR 1. Yes

Date asked about treatment other than CPR 11/4/2025

Was patient/responsible party asked about hospitalization 1. Yes

Date asked about hospitalization 11/4/2025

Was patient/responsible party asked about spiritual/existential concerns 1. Yes

Date asked about spiritual/existential concerns 11/4/2025

Active Diagnoses

Principal Diagnosis 04. Stroke

Comorbidities and Co-existing Conditions

☒ Heart Failure (e.g., congestive heart failure (CHF) and pulmonary edema)

☒ Cardiovascular (excluding heart failure)

Pain

Was patient screened for pain 1. Yes

Date of first screening for pain 11/4/2025

Patient's pain severity was 1. Mild

Type of standardized pain tool used 9. No standardized tool used

Is pain an active problem for the patient? 1. Yes

Was comprehensive pain assessment done 1. Yes

Date of comprehensive pain assessment 11/4/2025

Comprehensive pain assessment included:

☒ Location

☒ Severity

☒ Character

☒ Frequency

☒ Duration

☒ What relieves/worsens pain

☒ Effect on function or quality of life

Neuropathic Pain 1. Yes

Symptom Impact

Was a symptom impact screening completed? 1. Yes

Date of symptom impact screening 11/4/2025

Pain 1. Slight

Shortness of breath 1. Slight

Anxiety 1. Slight

Nausea 0. Not at all

Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	0. Not at all
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No
Was PRN opioid initiated or continued? 1. Yes
Date PRN opioid initiated or continued 11/4/2025
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 11/4/2025

TEACHING/EDUCATION PROVIDED

Education Provided To:

- ☒ Patient
- ☒ Caregiver

Topics

- ☒ Palliative care, hospice philosophy, hospice benefit
- ☒ Scope of services, role of team members
- ☒ Call us First Program
- ☒ Diagnosis
- ☒ Prognosis
- ☒ Disease process
- ☒ Safe & effective use of equipment & supplies
- ☒ Dying process, signs & symptoms of approaching death
- ☒ Covered medications, supplies, equipment
- ☒ How to re-order medications and supplies
- ☒ Medication usage, side effects, drug interactions
- ☒ Basic home safety, emergency preparedness
 - ☒ Provided written personalized emergency preparedness plan
 - ☒ Resides in a facility. Follow facility plan
- ☒ Infection control, disposal of hazardous waste, universal precautions
- ☒ Skin care, wound care, dressing changes
- ☒ Energy Conservation techniques
- ☒ Plan of care established with primary care giver
- ☒ Symptom management
- ☒ Catheter care
- ☒ Bowel regimen
- ☒ Pain management

- ☒ What to do at the time of death
- ☒ Anticipatory grief, grief, bereavement
- ☒ Funeral planning
- ☒ Advance directives
- ☒ Community resources
- ☒ Signs & symptoms of infection
- ☒ Discharge planning

COMMUNICATION WITH OTHER TEAM MEMBERS

- ☒ Hospice Medical Director
- ☒ Clinical Director/PCC
- ☒ Assigned RN Case Manager
- ☒ Assigned LPN
- ☒ Social Worker
- ☒ Volunteer Coordinator
- ☒ Bereavement Coordinator
- ☒ Spiritual Counselor

Date: 11/4/2025

Electronically signed by: TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

SPIRITUAL ASSESSMENT

SPIRITUAL ASSESSMENT

Date/Time in: 11/4/2025 2:46:00 PM
Date/Time out: 11/4/2025 3:14:00 PM
Location/address: Patient home

Response to Terminal Prognosis

- ☒ Patient unable to respond
 - Acceptance
 - ☒ Patient
 - ☒ Representative

Pain Assessment

Pain rating: 0
Pain scale used: Non-Verbal Scale

Indicators of Spiritual Strengths

Comments related to spiritual strengths:	Patient is unable to participate in spiritual conversation due to cognitive limitations. Family demonstrates adequate coping and strong support systems. No indicators of spiritual or emotional distress were noted. Family's decision to limit chaplain involvement reflects confidence in existing supports rather than resistance or distress.
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Indicators of Spiritual Concerns/Spiritual Distress

Teaching and other interventions provided during visit (include patient/family's response to teaching/interventions):	Chaplain made initial contact with the daughter of patient Sandra Hamlin following the patient's return home from the hospital and resumption of hospice services. The daughter was pleasant and receptive, expressing appreciation for the chaplain's call. She reported that the patient's condition has remained stable since discharge, with no major changes. Patient is described as bedbound, confused, and dependent on a paid caregiver for most daily activities. The caregiver shares responsibilities with the patient's sister-in-law, who is also on hospice and serves as her primary emotional companion. The family lives nearby, visits regularly, and coordinates together to ensure that the patient's needs are met. Family expressed that they are comfortable and coping well at this time. They continue to decline routine chaplain visits for themselves but remain open to chaplain support for the patient as needed. The daughter verbalized appreciation for the chaplain's availability and for the hospice team's ongoing care.
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Communication and Comments

Comments related to Plan of Care and overall impressions:	Chaplain will respect family's preference to decline regular visits while maintaining availability for emergent or patient-specific spiritual needs. Will continue to monitor through team updates and provide follow-up if family requests additional support.
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Date: 11/4/2025

Electronically signed by: DAMIEN DAWSON M.DIV

Patient: SANDRA HAMLIN
Medical Record: #90008

Bereavement Risk Assessment

Date/Time In	11/4/2025 3:15:00 PM
Date/Time Out	11/4/2025 3:30:00 PM
Risk Assessment Grid	
Caregiver	Susan Phillips (primary - Daughter)
1: Possible alcohol/substance abuse	☐
1: Neglect of appearance	☐
1: History of mental or emotional health concerns	☐
1: History of marked dependence on patient	☐
1: History of suicide ideation or intent	☐
1: Age less than 18	☐
1: Absence of meaningful spiritual support	☐
1: Death means loss of constant companion or emotional support	☐
1: Death will result in loss of financial provisions/financial support	☐
1: Death means change or loss of role or function	☐
1: Unable to share feelings	☐
1: Difficulty making decisions	☐
1: Ambivalent relationship with the patient	☐
1: Death means loss of meaning/purpose in life	☐
1: Communication difficulties	☐
1: Cultural issues present	☐
1: Inflexible thinking	☐
1: Dependent family members/children living in patient's home	☐
1: Unfinished business with patient	☐
1: Sense of hopelessness	☐
1: Signs of spiritual distress	☐
1: Estranged or isolated from support system	☐
1: Inadequate or negative coping skills	☐
1: Concurrent life crisis	☐
1: Multiple or recent losses	☐
1: Unprepared for death/strong denial prior to death	☐
1: Difficulty coping with past losses	☐
1: Unresolved grief from prior losses	☐
1: Disenfranchised grief	☐
1: History of family violence	☐
1: Family or marital conflict	☐
Risk Total:	
Comments	Patient POA has been caring for the patient for a long time with the help of 24hr caregivers. They are accepting of the hospice diagnosis, have a good support system and are grieving appropriately. Risk assessed as low.

Electronically signed by: DAMIEN DAWSON M.DIV

Patient: SANDRA HAMLIN

Medical Record: #90008

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 11/4/2025
Benefit Period: Second 90 Days
Certification Period - From: 11/4/2025
Certification Period - To: 2/1/2026
Primary Diagnosis: Cerebral infarction, unspecified
ICD-10 Code: I63.9

Written Certification: Based on my clinical judgment, I certify that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.

Physician Narrative: Explain the clinical findings that support a life expectancy of six months or less. If applicable, include an explanation of why the findings of the face-to-face encounter support a life expectancy of six months or less.
Note: This narrative must reflect the patient's individual clinical circumstances and cannot contain check boxes or standard language used for all patients.

Narrative: Pt is a 82 -year-old widowed female with a primary diagnosis of cerebral infarction, She presents with related conditions including unspecified pain , nausea with vomiting , gastro-esophageal reflux disease with esophagitis, without bleeding and unspecified protein-calorie malnutrition .Patient admitted to hospice with the following declines in the past 3 to 6 months Pt has a history of multiple hospital admissions for suspected cancer, CVA, MI, syncope, history of GT placement. She has right-sided weakness, severe hypertension, and an episode of syncope. Despite being sent home with private in-home care, Ms. Hamlin's condition has continued to decline, evidenced by a progressive weight loss of 11 pounds over the last two months, bringing her weight to 130 pounds and a BMI of 21. Her serum albumin level is notably low at 2.5 g/dL, indicating impaired nutritional status. Her Karnofsky Performance Status is 50%, and the Palliative Performance Scale (PPS) is also 50%, signifying moderate disease progression with significant care needs. Patient presents with a left MAC of 22 cm. Patient needing assistance with 6 out of 6 ADLs. Patient hospice appropriate life expectancy less than 6 months.

Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD

Date: 11/4/2025

Patient: SANDRA HAMLIN

Medical Record: #90008

HOSPICE PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Hospice Physician Initial Certification of Terminal Illness

Start of Care Date: 11/4/2025
Benefit Period: Second 90 Days
Certification Period - From: 11/4/2025
Certification Period - To: 2/1/2026
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Attestation: By signing, I confirm that I composed the narrative based on my review of the patient's medical record or, if applicable, my examination of the patient.

Electronically signed by: DIEGO F RESTREPO MD

Date: 11/4/2025

Patient: SANDRA HAMLIN
Medical Record: #90008

VERBAL PHYSICIAN INITIAL CERTIFICATION OF TERMINAL ILLNESS

Verbal Physician Initial Certification of Terminal Illness

Start of Care Date: 11/4/2025
Benefit Period: Second 90 Days
Certification Period - From: 11/4/2025
Certification Period - To: 2/1/2026
Primary Diagnosis: Cerebral infarction, unspecified
ICD-10 Code: I63.9

Hospice Physician Initial Certification of Terminal Illness

Verbal Certification:
On 11/4/2025
Dr. DIEGO F RESTREPO MD
certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.
Verbal Certification Received By:
Electronically signed by: TRACY KING RN
Date: 11/4/2025

Physician Signature / Date:

Attending Physician Initial Certification of Terminal Illness

Verbal Certification:
On 11/4/2025
Dr. DIEGO F RESTREPO MD
certified that this patient is terminally ill with a life expectancy of 6 months or less if the terminal illness runs its normal course.
Verbal Certification Received By:
Electronically signed by: TRACY KING RN
Date: 11/4/2025

Physician Signature / Date:

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Sandra Hamlin, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% – Medicare Beneficiary Identifier (MBI): 7TT4-UP3-MP93 Effective Date: 10/01/2003

☐ Bill Medicaid 100% – Medicaid Beneficiary Number: _____ Effective Date: _____

☐ Bill Primary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

☐ Bill Secondary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

Benefits are: _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST

☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives Information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

Beneficiary/Patient Signature

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:25 CDT

Date

October 30, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

POA

Cognitive Decline due to Advanced DX

Relationship to Patient

Reason patient is unable to sign

E-SIGNED by Jomya Pollard
on 2025-10-30 10:48:30 CDT

October 30, 2025

Hospice Agency Representative Signature

Date

Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Sandra Hamlin, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 11/4/2025 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - Provided by the designated Hospice.
 - Provided by another hospice under arrangements made by the designated Hospice.
 - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☒ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) 1063664076

Office Address: _____

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☒ N/A Patient is non-Medicare

☒ N/A I was transferred from _____ on _____.

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Homage Hospice _____ with the effective date of election to begin on 11/4/2025.

I am electing the (1st, 2nd, 3rd, 4th, etc.) 3rd benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

Beneficiary/Patient Signature

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:33 CDT

Date

October 30, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

POA

Cognitive Decline Due to Advanced DX

Relationship to Patient

Reason patient is unable to sign

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:32 CDT

October 30, 2025

Hospice Agency Representative Signature

Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs

Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:36 CDT

October 30, 2025

Signature of Beneficiary/Patient's Authorized Representative

Date

POA/Cognitive Decline Due to Advanced DX

Representative's Relationship/Reason Patient Unable to Sign

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:33 CDT

October 30, 2025

Signature of Witness/Hospice Representative

Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Sandra Hamlin.
I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

- I. I understand the Hospice's obligations are as follows:
 - A. The Hospice staff will provide instructions to me as needed.
 - B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.
 - C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.
- II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.
- III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:39 CDT

Caregiver Signature

October 30, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:35 CDT

Hospice Signature

October 30, 2025

Date

No Primary Caregiver Agreement

I, Sandra Hamlin, do agree and will comply with the following:

- Understand that I have an illness which has been diagnosed as appropriate for hospice care.
- Understand the goal of hospice care is to relieve, control or minimize symptoms to maintain optimal quality of life in patients with a life limiting illness. Treatment and diagnostic procedures which would cause increased discomfort without increased quality of life will be avoided.
- At the present time I live alone with no primary caregiver.
- When the Hospice Team determines that, due to disease progression or changes in my condition, it is unsafe for me to remain in my home alone, I agree to the following change of my environment. This may include contacting or hiring someone to provide 24-hour care to me.

1. _____
2. _____
3. _____
4. _____

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:42 CDT

October 30, 2025

Patient Signature

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:36 CDT

October 30, 2025

Witness

Date

Patient Choice Statement
(To be completed for all new patients/guardians)

I, Sandra Hamlin (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Homage Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:45 CDT

October 30, 2025

Signature of Patient/Guardian

Date

E-SIGNED by Jorjya Pollard
on 2025-10-30 10:48:38 CDT

October 30, 2025

Signature of Hospice Representative

Date

Hospice Wound Photography Consent

I, Sandra Hamlin _____ hereby consent and so authorize the taking
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 11-4-2025

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:49 CDT

October 30, 2025

Patient Signature (or other authorized signature)

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:39 CDT

October 30, 2025

Witness Signature

Date

POA

Relationship, if other than patient

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:52 CDT

Beneficiary/Patient/Authorized Representative Signature

October 30, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:41 CDT

Hospice Agency Representative Signature

October 30, 2025

Date



HOMAGE
—HOSPICE—

Homage Hospice
8204 Elmbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name Sandra Hamlin Birth Date 09/26/1943
Address 5813 Excalibur Dr
City Garland State TX Zip 75044
Home Phone _____ Business Ph _____ Cell 972-849-7060

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

****Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.****

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:
Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Hospice Tools in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:55 CDT

October 30, 2025

Patient/Authorized Signature

Date

Medicare Secondary Payer Screening Form

Patient Name:

Hamlin, Sandra

SOC:

11-4-2025

Date:

11-4-2025

PART I

- Are you receiving Black Lung (BL) Benefits?
☒ No Go to 2.
☐ Yes BL is primary payer only for claims related to BL. Date benefits began: _____
- Are the services to be paid by a government research program?
☒ No Go to 3.
☐ Yes Government Research Program will pay primary benefits for these services.
- Has the Department of Veterans Affairs (DVA) authorized and agreed to pay for your care at this facility?
☒ No Go to 4.
☐ Yes DVA is primary for these services.
- Was the illness/injury due to a work-related accident/condition?
☒ No Go to Part II
☐ Yes Go to *Insurance Information Form* WC is primary payer only for related claims for work-related

PART II

- Was illness/injury due to a non-work-related accident?
☒ No Go to Part III
☐ Yes Go to *Insurance Information Form*

PART III

- Are you entitled to Medicare based on:

☒ Age- Go to PART IV

☐ Disability- Go to PART V

☐ End-Stage Renal Disease (ESRD)- Go to PART VI.

Complete ALL "PARTS" associated with the patient's selections.

PART IV - AGE

If the patient answers "no" to 2, Medicare is primary unless answered "yes" to questions in part I or II.

- Are you or your spouse currently employed?
Patient ☒ No - Never worked ☐ No - Retired Date: _____ ☐ Yes Go to 2.
Spouse ☒ No - Never worked ☐ No - Retired Date: _____ ☐ Yes Go to 2.
- Do you have group health plan (GHP) coverage based on your own or a spouse's current employer who employs 20 or > employees?
Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*

PART V - DISABILITY

If the patient answers "no" to all, Medicare is primary unless answered "yes" to questions in part I or II.

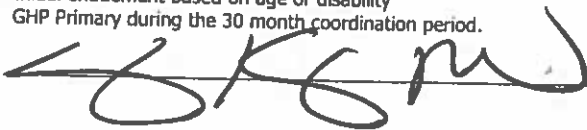
- Do you have group health plan (GHP) coverage based on your own, a spouse or family member's current employer who employs 100 or > employees?
Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Family ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*

PART VI - ESRD

If the patient answers "no" to all in 1, Medicare is primary unless answered "yes" to questions in part I or II.

- Do you have group health plan (GHP) coverage based on your own, a spouse or family member?
Patient ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Spouse ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
Family ☒ No ☐ Yes, GHP is primary Go to *Insurance Information Form*
- Have you received a kidney transplant?
☒ No
☐ Yes Date of Transplant: _____
- Have you received maintenance dialysis treatments?
☒ No
☐ Yes Dialysis Start Date: _____ If app, Self-Dialysis Training Program Start Date: _____
- Are you within the 30-month coordination period that starts MM/DD/CCYY? (The 30-month coordination period starts the first day of the month an individual is eligible for Medicare (even if not yet enrolled in Medicare) because of kidney failure (usually the fourth month of dialysis). If the individual is participating in a self-dialysis training program or has a kidney transplant during the 3-month waiting period, the 30-month coordination period starts with the first day of the month of dialysis or kidney transplant.)
☒ No Medicare is Primary
☐ Yes
- Are you entitled to Medicare on the basis of either ESRD and age or ESRD and disability?
☒ No
☐ Yes
- Was your initial entitlement to Medicare (including simultaneous or dual entitlement) based on ESRD?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.
- Does the working aged or disability MSP provision apply (i.e., is the GHP already primary based on age or disability entitlement)?
☒ No Initial entitlement based on age or disability
☐ Yes GHP Primary during the 30 month coordination period.

Hospice Signature:



Date:

11-4-2025

Hospice Informed Consent/Election of Hospice Benefit

Hospice Agency: Homage Hospice

Hospice 24-Hour Number: 469-625-0705

Informed Consent for Hospice Service/Release of Records (Part One of Two Requires Signatures)

I, Sandra Hamlin, have been informed that the above named Hospice Agency, referred to in this consent as the Hospice, offers hospice care under the Medicare/Medicaid Hospice Benefit Program. I understand the following explanation of the Medicare/Medicaid Hospice Benefit.

Admission Information: The Hospice will admit you only if the Hospice is able to provide care appropriate to your needs. If the Hospice is unable to meet your needs, the Hospice will assist you and your representative in locating resources of your choice that can provide the needed services.

After Hours Access/Inpatient Care: A nurse is scheduled to respond after hours to patient calls, while the Hospice's office is closed. If a nurse is needed after hours, call the Hospice's phone number provided above. The call will either be answered by a nurse or routed to a nurse via an answering service.

If inpatient palliative support is needed, the Hospice will provide and coordinate inpatient care at a contracted facility. I understand that if I call 911, go to the hospital, or choose care for any conditions related to my admitting hospice diagnosis without preauthorization from the Hospice, I may be responsible for those associated costs.

Clinical Records: It is the policy of the Hospice to protect all clinical records against loss, defacement, tampering, and use by unauthorized persons. I authorize the Hospice to release medical information to my physician, the facility of my choice, payer source, or accrediting/regulatory/consulting organizations, as appropriate.

If I transfer to another healthcare entity or facility, the Plan of Care, discharge summary, and medical record, if requested, may be released to the new service providers.

Financial Authorization

I authorize benefits to be made on my behalf.

☒ Bill Medicare 100% – Medicare Beneficiary Identifier (MBI): 7TT4-UP3-MP93 Effective Date: 10/01/2003

☐ Bill Medicaid 100% – Medicaid Beneficiary Number: _____ Effective Date: _____

☐ Bill Primary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

☐ Bill Secondary Insurance: _____ % Insurance Co: _____

Beneficiary/Pre-Auth Numbers: _____ Effective Date: _____

Benefits are: _____

☒ I will pay any service or supply charge not reimbursed by my insurance company on a monthly basis. I will pay all charges incurred on a monthly basis if I do not have insurance coverage.

Hospice Informed Consent/Election of Hospice Benefit Services/Rights/Hotline/Policies/Procedures

I understand that a Registered Nurse (RN) will case manage all services, and I have accepted the following services:

☒ RN and/or LVN/LPN ☒ Hospice aide ☒ Social worker ☒ Chaplain ☐ Dietitian ☐ Volunteer ☐ PT ☐ OT ☐ ST

☐ Other: _____

I have been notified of my right to voice a complaint and may direct that to the Hospice Administrator, or designee, at the above referenced phone number. The investigation of the complaint will be initiated within 10 calendar days and resolved within 30 calendar days of receipt. I may also contact Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake online through the Texas Unified Licensure Information Portal (TULIP) at <https://txhhs.force.com/complaint/s/>; or by calling 1-800-458-9858 (Monday through Friday, 7am to 7pm). My complaint may also be mailed to Texas Health and Human Services Commission (HHSC), Complaint and Incident Intake, Mail Code E249, P.O. Box 149030, Austin, Texas 78714-9030 or faxed to: 1-877-438-5827 (toll-free) or to 1-512-438-2724. This includes a complaint regarding advance directives.

I can also file a complaint at Medicare.gov or call the Medicare Beneficiary Ombudsman's office at 1-800-MEDICARE (1-800-633-4227, or either 1-877-486-2048 or 711 for TTY users). Representatives are available 24 hours a day, seven days a week. If I still need help after talking with a representative, I can ask to have my inquiry referred to the Ombudsman. For any complaints regarding discrimination, I understand that I may contact the Office for Civil Rights within 180 days of when the situation occurred. The complaint must be filed in writing by mail, fax, email, or online via the OCR Complaint Portal.

Complaints regarding any health insurance services can be mailed to MC 1111A, Consumer Protection, Texas Department of Insurance, PO Box 149091, Austin, Texas 78714, or by calling the Consumer Help Line at 1-800-252-3439, online at <http://www.tdi.texas.gov/consumer/complfrm.html>, or by email to ConsumerProtection@tdi.texas.gov.

I acknowledge I have received a copy along with a verbal explanation of the following:

- Patient Rights and Responsibilities and Patient Choice Statement
- Rights of the Elderly
- Advance Directives Information
- HIPAA/Notice of Privacy Practices information
- Abuse, Neglect, and Exploitation policy
- Emergency Preparedness/Natural Disaster information
- Home Safety information
- Disposal of Controlled Substances and Medication Disposal policies
- Medicare Part D information
- Home Hazardous Waste Disposal information
- Infection Control education
- Hospice Drug Testing policy
- Expected Death (at Home) policy

I have received education on completing an emergency preparedness plan for myself and my family. I understand the importance of completing this plan and know that the Hospice staff may assist in this process.

Hospice Informed Consent/Election of Hospice Benefit

I understand that the Hospice will review all of my medications to determine if they are related to my hospice diagnosis and related conditions to determine whether medications will be covered by hospice or my Part D plan.

I consent to the Hospice's use and/or disclosure of protected health information for payment, treatment, and the Hospice's healthcare operations.

I have been informed of the availability of clergy and spiritual counseling services.

I understand that it is my right and responsibility to be involved in my care and that I will be informed as to the nature and purpose of any procedures. I also understand that I must have a primary caregiver.

I have ☐ or have not ☐ signed an advance directive. I am ☐ am not ☐ providing a copy for my record.

Health Care Proxy/Agent/Surrogate/Attorney in Fact: _____ Phone: _____

Notice of Service Not Covered by Hospice

The following services will not be covered by the Hospice:

- Conditions, medications, and services that are not related to the terminal diagnosis and related conditions.
- A hospitalization in a facility that does not contract with the Hospice.
- Hospitalization without contacting and arranging with the Hospice's nurse.
- Ambulance use without the Hospice's approval.
- Physician appointment without notifying the Hospice's nurse.
- Curative or life extending therapies such as blood or blood products, chemotherapy, radiation, or insertion of PEG feeding tube.
- Anything not related to the Hospice diagnosis and related conditions.
- Sitter services.

The above services are not covered by hospice. The Hospice is reimbursed through Medicare, Medicaid, private insurance, or private pay. This is done on a per diem or per visit basis or schedule.

Due to this, the Hospice must be financially responsible for care provided regarding the terminal diagnosis and related diagnoses and related needs of the Hospice patient. The Hospice Medical Director will determine which conditions are related to the Hospice life-limiting terminal diagnosis.

Medicare guidelines instruct hospices to be professional managers of the patient's care. To do this, the Hospice must be aware of the situations as they arise and be allowed to manage the care of the patient. If the patient/representative desire any of the above non-covered items, the patient/representative will be given a choice of either reinstating the patient's traditional Medicare services and discontinuing hospice services or allowing the patient/representative to pay for the non-covered services.

I must seek pre-approval from the Hospice for all treatments and services not included in the Plan of Care. I will be responsible for all charges incurred for treatments/services with a physician or facility not contracted with the Hospice. I have received a list of all inpatient and respite facilities with which the Hospice has a contract.

Hospice Informed Consent/Election of Hospice Benefit

Required Signatures for the Hospice Informed Consent (Part One of Two Signatures)

Beneficiary/Patient Signature

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:25 CDT

Date

October 30, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

POA

Cognitive Decline due to Advanced DX

Relationship to Patient

Reason patient is unable to sign

E-SIGNED by Jomya Pollard
on 2025-10-30 10:48:30 CDT

October 30, 2025

Hospice Agency Representative Signature

Date

Election of the Medicare/Medicaid Hospice Benefit (Part Two of Two Requires Signatures)

I, Sandra Hamlin, choose to elect the Medicare Hospice Benefit and receive hospice services from the above named Hospice Agency (the Hospice) to begin on 11/4/2025 (start of care date).

(Note: The start of care date, also known as the effective date of the election, may be the first day of hospice care or a later date, but may be no earlier than the date of the election statement. An individual may not designate an effective date that is retroactive.)

Hospice Philosophy and Effects of Electing the Medicare Hospice Benefit

I acknowledge that I have been given a full explanation and have an understanding of the purpose of hospice care. Hospice is a holistic, comprehensive type of care for those facing a life-limiting terminal illness. Hospice care concentrates on managing pain and other symptoms related to my terminal illness and related conditions. I acknowledge and understand that hospice care is palliative rather than curative, and such care will not be directed toward cure. The focus of hospice care is to provide comfort and support to me and my family/caregivers/representatives. I also acknowledge that I have been given a full understanding of the potential availability of supportive palliative care options outside a hospice setting.

I understand that by electing hospice care under the Medicare hospice benefit, I waive (give up) the right to Medicare payments for items, services, and drugs related to my terminal illness and related conditions. This means that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I have received information on which items, services, and drugs the Hospice will cover and furnish upon my election to receive hospice care, and I understand that while this election is in force, Medicare will make payments for care related to my terminal illness and related conditions only to the designated hospice and attending physician that I have selected.

I understand that items, services, and drugs unrelated to my terminal illness and related conditions are exceptional and unusual, and in general the Hospice will be providing virtually all of my care needed while I am under a hospice election.

I understand that items, services, and drugs determined to be unrelated to my terminal illness and related conditions continue to be eligible for coverage by Medicare under separate benefits.

I acknowledge that I have been provided information that the cost sharing for hospice services is minimal and that prescription drugs and inpatient respite care are the only services potentially subject to cost sharing. Hospices may charge coinsurance of five percent

Hospice Informed Consent/Election of Hospice Benefit

for each prescription provided outside the inpatient setting (not to exceed \$5.00) and for inpatient respite care (not to exceed the inpatient hospital deductible).

Right to Request "Patient Notification of Hospice Non-Covered Items, Services, and Drugs"

As a Medicare beneficiary electing to receive hospice care, I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the notification within five days of the Hospice election date, the Hospice will provide the information within five days of my request. If I request the notification at any point after the first five days of the Hospice election date, the Hospice will provide this form within three days of my request.

Beneficiary and Family-Centered Care Quality Organization (BFCC-QIO)

I acknowledge that as a Medicare beneficiary, I have been provided information regarding the provision of Immediate Advocacy through the Beneficiary and Family Centered Care Quality Organization (BFCC-QIO) if I disagree with any of the Hospice's determinations, and that I have been provided with the contact information for the BFCC-QIO that services my area, which is Acentra Health: www.acentraqio.com, phone number 888-315-0636, TTY number 711.

I understand that I may revoke my election of the Hospice benefit at any time during a certification period, and thereby resume the Medicare or Medicaid coverage of the benefits I waived. To revoke the election, I understand I must file a written revocation statement with this Hospice Agency. At a later date, I may elect to receive hospice coverage again for the next certification period if I am eligible.

Waiver of Other Benefits

For the duration of an election of hospice care, I waive all rights to Medicare payments for the following services:

- Hospice care provided by a hospice other than the Hospice designated by me (unless provided under arrangements made by the designated Hospice).
- Any Medicare services that are related to the treatment of my terminal condition for which hospice care was elected or a related condition or that are equivalent to hospice care except for services:
 - Provided by the designated Hospice.
 - Provided by another hospice under arrangements made by the designated Hospice.
 - Provided by the individual's attending physician if that physician is not an employee of the designated Hospice or receiving compensation from the Hospice for those services.

Right to Choose Attending Physician

I understand that I have a right to choose my attending physician to oversee my care if available and agrees to serve in this role. My attending physician will work in collaboration with the Hospice to provide care related to my terminal illness and related conditions.

☒ I do not wish to choose an attending physician.

I acknowledge this choice is of my own free will without coercion. My choice for an attending physician is:

(Please provide any information that will uniquely identify your attending physician choice.)

Physician Full Name: Dr. Diego Restrepo NPI (if known) 1063664076

Office Address: _____

Hospice Informed Consent/Election of Hospice Benefit

If my chosen attending physician declines to serve in that role, I understand that the Hospice's physicians will manage my medical care related to my terminal illness and related conditions unless I choose a different attending physician.

Required Signatures for the Election of the Hospice Benefit (Part Two of Two Signatures)

☒ N/A Patient is non-Medicare

☒ N/A I was transferred from _____ on _____.

I acknowledge and understand the above, and authorize Medicare hospice coverage to be provided by (the Hospice)

Homage Hospice

with the effective date of election to begin on 11/4/2025.

I am electing the (1st, 2nd, 3rd, 4th, etc.) 3rd benefit period.

Note: The start of care date, also known as the effective date, of the election, which may be the first day of hospice care or a later date but may be no earlier than the date of the election statement as indicated above. An individual may not designate an effective date that is retroactive.

Beneficiary/Patient Signature

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:33 CDT

Date

October 30, 2025

Authorized Beneficiary Representative Signature (if beneficiary is unable to sign)

Date

POA

Cognitive Decline Due to Advanced DX

Relationship to Patient

Reason patient is unable to sign

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:32 CDT

October 30, 2025

Hospice Agency Representative Signature

Date

Patient Declination / Request for Notification of Hospice Non-Covered Items, Services, and Drugs

Homage Hospice

I understand that I have the right to request at any time, in writing, the "Patient Notification of Hospice Non-Covered Items, Services, and Drugs" addendum that lists conditions, items, services, and drugs that the Hospice has determined to be unrelated to my terminal illness and related conditions, and that will not be covered by the Hospice.

If I request the addendum within five days of the Hospice election date, the Hospice must provide the form within five days of the request. If I request the addendum at any point after the first five days of the Hospice election date, the Hospice must provide the notification within three days of my request. Additionally, if there are changes to the Hospice Plan of Care after the original request that indicate a new illness or condition has arisen, the Hospice will issue an updated addendum to me (or my representative) reflecting whether or not items, services, and supplies related to the new illness or condition will be provided by the Hospice.

I am requesting, in writing, that the Hospice furnish a copy of the above-described addendum. My written request is set forth on this date: _____. The following box checked indicates the point in time I submitted the written request to the Hospice, and how long the Hospice has in turn to furnish the requested addendum.

☐ I submit my request in writing within five days of the Hospice election date then I expect my request to be fulfilled within five days of the written date above.

Or

☐ I submit my request in writing at any point after the first five days of the Hospice election date then I expect my request to be fulfilled within three days of the written date above.

☒ I am not requesting the addendum. I have been made aware that I can request it at any time.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:36 CDT

October 30, 2025

Signature of Beneficiary/Patient's Authorized Representative

Date

POA/Cognitive Decline Due to Advanced DX

Representative's Relationship/Reason Patient Unable to Sign

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:33 CDT

October 30, 2025

Signature of Witness/Hospice Representative

Date

The Hospice furnished the addendum on:

Date

Primary Care Person Responsibilities

I understand that I am accepting the role as the primary care person for Sandra Hamlin.

I further understand this will involve receiving support from this Hospice. The commitment and responsibility of the Hospice is described below. I have a commitment to keep my loved one at home during his/her terminal illness.

I also have a commitment to provide for his/her care as much and as long as I am able to do so.

I understand the Hospice's goal is not to cure a patient's terminal illness, but to reduce symptoms such as pain and nausea and to provide psycho-social and spiritual support to both the patient and the family.

- I. I understand the Hospice's obligations are as follows:
 - A. The Hospice staff will provide instructions to me as needed.
 - B. The Hospice staff will provide emotional and spiritual support to help me cope with this illness and eventual death. This support will be provided by hospice staff and may include volunteers. Hospice staff will also provide support to me after death through a bereavement program.
 - C. The Hospice staff will provide patient care by scheduled appointments. Visits will be made to the home and/or nursing facility.
- II. I understand the Hospice is available 24 hours a day, and I may call after hours for emergencies.
- III. I understand that because the patient and family are cared for as one, the Hospice medical record will contain information about me as well as the patient. Every effort will be made to keep this information confidential. Information about me will be exchanged with the patient.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:39 CDT

Caregiver Signature

October 30, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:35 CDT

Hospice Signature

October 30, 2025

Date

No Primary Caregiver Agreement

I, Sandra Hamlin, do agree and will comply with the following:

- Understand that I have an illness which has been diagnosed as appropriate for hospice care.
- Understand the goal of hospice care is to relieve, control or minimize symptoms to maintain optimal quality of life in patients with a life limiting illness. Treatment and diagnostic procedures which would cause increased discomfort without increased quality of life will be avoided.
- At the present time I live alone with no primary caregiver.
- When the Hospice Team determines that, due to disease progression or changes in my condition, it is unsafe for me to remain in my home alone, I agree to the following change of my environment. This may include contacting or hiring someone to provide 24-hour care to me.

1. _____
2. _____
3. _____
4. _____

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:42 CDT

October 30, 2025

Patient Signature

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:36 CDT

October 30, 2025

Witness

Date

Patient Choice Statement
(To be completed for all new patients/guardians)

I, Sandra Hamlin (patient/guardian name), the undersigned patient/guardian, understand that it is my right to select a hospice provider of my choice.

I have selected Homage Hospice (hereafter, the Hospice) free of any undue pressure or solicitation by any officer, director, employee, agent, or contractor of the Hospice. I have been advised by the Hospice that I may request information concerning its scope of practice, services available and telephone numbers. I was encouraged to ask specific questions concerning my individual needs to assess whether the Hospice is a good fit for me. I have been able to ask questions and express concerns, which have been satisfactorily responded to by the Hospice's staff. I further declare that my receipt of hospice services from the Hospice is by choice. I have been advised by the admitting professional that if for any reason I wish to change services to another hospice agency, it is my right to do so.

I am making this request of my own free will and have not been coerced, solicited, or pressured to do so by any employee of the Hospice.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:45 CDT

October 30, 2025

Signature of Patient/Guardian

Date

E-SIGNED by Jorjya Pollard
on 2025-10-30 10:48:38 CDT

October 30, 2025

Signature of Hospice Representative

Date

Hospice Wound Photography Consent

I, Sandra Hamlin hereby consent and so authorize the taking
(Name of Patient)

of photographs by a representative of the Hospice. I understand that all photographs taken will be used solely for purposes of medical information and that they will be treated and handled in a confidential manner. Further, I understand that I may request possession of the photographs or that they be destroyed at any time.

I further hereby release the Hospice and its personnel from any and all liability in the taking and use of these photographs. I also understand that these photographs may be submitted upon request to insurance companies, Medicare or Medicaid, and other payors of care.

This consent is entered into on the following date: 11-4-2025

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:49 CDT

October 30, 2025

Patient Signature (or other authorized signature)

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:39 CDT

October 30, 2025

Witness Signature

Date

POA

Relationship, if other than patient

Patient Acknowledgment of Initial Nondiscrimination Notices

Hospice Conditions of Participation 418.52 and 418.104(a)(2) requires the Hospice to provide a notice of patient rights and have a signed copy of the notice. Federal civil rights regulations require the Hospice to provide participants, beneficiaries, enrollees and applicants of its Hospice program and activities, the following notices:

Notice of Nondiscrimination

- Informs individuals about nondiscrimination and accessibility requirements.

Notice of Availability of Language Assistance Services and Auxiliary Aids and Services

- Informs individuals about the availability of free language assistance services, auxiliary aides and services to provide information in accessible formats.

By my signature below, I confirm that I have received a copy along with a verbal explanation of the additional patient rights notices listed above in a language I understand. I am also aware if I need modifications for effective communication, those modifications are available to me as necessary at no cost.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:52 CDT

Beneficiary/Patient/Authorized Representative Signature

October 30, 2025

Date

E-SIGNED by Jomya Pollard
on 2025-10-30 10:49:41 CDT

Hospice Agency Representative Signature

October 30, 2025

Date



HOMAGE
—HOSPICE—

Homage Hospice
8204 Elmbrook Drive, Suite 276
Dallas, Texas 75247

Office Hours:
Mon - Fri.: 8:30 a.m. - 5:00 p.m.

Phone: (469) 625-0705

eFax: (469) 617-6998

Medical Records Release Form

Patient Name Sandra Hamlin Birth Date 09/26/1943

Address 5813 Excalibur Dr

City Garland State TX Zip 75044

Home Phone _____ Business Ph _____ Cell 972-849-7060

Records Release Authorization

By my signature, I authorize Homage Hospice to receive my pertinent medical records for the patient identified by the information listed above.

Medical Records Requested

For the past 1 year:

****Radiology (Xray, Ultrasound, CAT Scan, MRI, Special tests) Reports, EKG, Specialist Consults, Hospitalization Summaries, Labs, Updated Medication List.****

I authorize the release of photocopies of the following medical records to Homage Hospice, its employees, and/or agents. For the purpose hereof, "Medical Records" includes the following:

Confidential HIV and communicable disease-related information (A.R.S. Section 36-661)
Confidential Alcohol & Drug Abuse-related information (42 CFR Section 2.1 ET SEQ)
Confidential Genetic Testing Information (A.R.S. Section 12-2801)

I have given my consent freely and without coercion. I may revoke this consent at any time by notifying Hospice Tools in writing. A photocopy of Facsimile of this authorization can substitute for the original. FAX RECORDS TO: (469) 617-6998.

E-SIGNED by Suzan Phillips
on 2025-10-30 14:41:55 CDT

October 30, 2025

Patient/Authorized Signature

Date



1750 Founders Parkway Suite 130
Alpharetta, GA 30009

855-681-8243 (Toll Free)
214-345-8811 (FAX)
www.verisma.com

Date: 6/11/2025

Fax: 469-617-6998

To:

HOMAGE HOSPICE
8204 ELMBROOK DRIVE
SUITE 276
DALLAS, TX 75247
Phone: 469-625-5060

Re:

HAMLIN, SANDRA
DOB: 09/26/1943
VSI ID: 7456-35730
Case #:

**Records
From:**

Electronic
612 E Lamar Blvd
Ste 100
Arlington, TX 76011

Pages in this distribution (including this cover sheet): 15

*Please call Customer Service at **855-681-8243** if you experience problems with the receipt of this information.*

Requestor Satisfaction Survey

Thank you for requesting medical records. To better serve you in the future, please take a brief survey of your experience at: <https://www.surveymonkey.com/r/verismasurvey1>

STATEMENT OF CONFIDENTIALITY

The information contained is intended for the exclusive use of the addressee and contains confidential or privileged information. If you are not the intended recipient, you are hereby notified that any retention, dissemination, or use of this communication is strictly prohibited. If this information was sent in error, please notify us by phone at the number listed above.

Mon Jun 9th, 2025 5:39 PM Central Time

FAX

Homage Hospice
Phone: 469-625-0705
Fax: 469-617-6998

TO:

Name: Medical Records Request

Fax Number: (214) 345-8811

of Pages: 2

(including cover sheet)

FROM:

Name: Intake | Homage Hospice

Fax Number: (469) 617-6998

Subject: Patient: Sandra Hamlin

Message:

Please send the most recent H&P, Physician notes, Progresss Notes, Labs, X-rays, Medication listing to the fax number above for Patient: Sandra Hamlin.

The information contained in this message, including attachments, is confidential and is intended for the addressee(s) only. If you are not the intended recipient or his or her authorized agent, you are notified that any use, distribution or copying of the message is strictly prohibited and could violate state and Federal laws, including the Health Insurance Portability and Accountability Act (HIPAA). If you have received this email in error, please notify the sender by replying to this message and delete this email immediately.

Medical Records Release Form

Client Name: Sandra R. Hamlin
Address: 5813 Escaliber Drive
City: Garland State: TX. Province:
Country: Zip/Postal Code: 75044
Telephone: Fax: Email:
Date of Birth: 09-26-1943 Social Security Number: 458-72-8838

I authorize the release of my medical records or other health care information, including intake forms, chart notes, reports, correspondence, billing statements, and other written information concerning my health and treatment during the period of 1/1/2024 to ; to be sent to the following person or company.

Company: Homage Hospice
Address: 8204 Elmbrook Drive, Suite 276
City: Dallas State: Texas Province:
Country: USA Zip/Postal Code: 75247
Telephone: 469-625-5060 Fax: 469-617-6998 Email: intake@homagehospice.com

Client Signature: [Signature] Date: 5-27-25

This authorization is valid until:

date

01/27/2024 - ED in Texas Health Dallas Emergency Department

Visit Information

Admission Information

Arrival Date/Time:	01/27/2024 11:14 AM	Admit Date/Time:	01/27/2024 11:19 AM	IP Adm. Date/Time:	
Admission Type:	Emergency	Point of Origin:	Non-health Care Facility	Admit Category:	
Means of Arrival:		Primary Service:	Emergency Room	Secondary Service:	N/A
Transfer Source:		Service Area:	SA-TEXAS HEALTH RESOURCES	Unit:	Texas Health Dallas Emergency Department
Admit Provider:		Attending Provider:	Perritt, Dirk, MD	Referring Provider:	

ED Disposition

ED Disposition	Condition	User	Date/Time	Comment
Discharge Home	Good	Perritt, Dirk, MD	Sat Jan 27, 2024 12:39 PM	--

Discharge Information

Date/Time: 01/27/2024 4:23 PM	Disposition: Home Or Other Long-term Residential Arrangement	Destination: ---
Provider: Perritt, Dirk, MD	Unit: Texas Health Dallas Emergency Department	

Visit Account Information

Hospital Account

Name	Acct ID	Class	Status	Primary Coverage
Hamlin, Sandra	20101540475	Emergency	Closed	MEDICARE - MEDICARE PART A AND B

Guarantor Account (for Hospital Account #20101540475)

Name	Relation to Pt	Service Area	Active?	Acct Type
Hamlin, Sandra	Self	TH	Yes	Personal/Family

Address	Phone
5813 EXCALIBUR DR GARLAND, TX 75044-3633	

Coverage Information (for Hospital Account #20101540475)

1. MEDICARE/MEDICARE PART A AND B

F/O Payor/Plan	Precert #
MEDICARE/MEDICARE PART A AND B	

Subscriber	Subscriber #
Hamlin, Sandra	7TT4UP3MP93

Address	Phone
PO BOX 3113 MECHANICSBURG, PA 17055-1828	

2. PHYSICIAN'S MUTUAL/PHYSICIAN'S MUTUAL

F/O Payor/Plan	Precert #
PHYSICIAN'S MUTUAL/PHYSICIAN'S MUTUAL	

Subscriber	Subscriber #
Hamlin, Sandra	3000695449

Address	Phone
PO Box 3313 Omaha, NE 68103-0313	888-747-1944

Texas Health Dallas
8200 Walnut Hill LaneHamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024

Printed by BY652WX at 6/11/25 4:44 PM

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)**Reason for Visit****Chief Complaint** [last edited by Helbing, Angela M, RN on 1/27/2024 1120]

- NG Tube Problem/Evaluation (Pt pulled out GI tube this morning. Replacement needed.)

Visit Diagnosis [last edited by Perritt, Dirk, MD on 1/27/2024 1239]

- Complaint associated with gastric tube (HCC) (primary)

ED Provider Notes**ED Provider Notes by Perritt, Dirk, MD at 1/27/2024 1120**

Author: Perritt, Dirk, MD

Filed: 01/28/24 1639

Status: Signed

Procedure Orders

Service: Emergency Room

Date of Service: 01/27/24 1120

Editor: Perritt, Dirk, MD (Physician)

Author Type: Physician

Creation Time: 01/27/24 1120

1. FEEDING TUBE REPLACEMENT [1418094771] ordered by Perritt, Dirk, MD

THDMGER ROOM#: ED14**Chief Complaint**

Patient presents with

- NG Tube Problem/Evaluation
Pt pulled out GI tube this morning. Replacement needed.

First Provider Contact Date and Time: 01/27/24 1126**History of Present Illness**

11:20 AM Sandra Hamlin is a 80 y.o. female with hx of CHF, HTN, HLD, and MI who presents to ED via EMS after pulling out her own GI tube this morning. EMS reports that pt pulled out her own G-tube but know nothing further. EMS reports stable vitals en route. HPI Limited because pt is nonverbal.

Independent history obtained from:

EMS

Prior records reviewed:

Prior DISCHARGE SUMMARY reviewed.

Prior visits from SAME ED reviewed.

Previous History**Past Medical History:**

Diagnosis

Date

- Anxiety and depression
- Asthma
as a child
- Broken heart syndrome
- Cardiomyopathy (HCC)
- Cataract
bilat extraction
- Chicken pox
- Chronic anemia
- Congestive heart failure (HCC)
- Coronary artery disease

2013

Texas Health Dallas
8200 Walnut Hill LaneHamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024

Printed by BY652WX at 6/11/25 4:44 PM

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

- CVA (cerebral infarction) 2000
- Diabetes mellitus (HCC) dx'd at 40 years old
- iddm
- Diverticular disease
- Essential hypertension, benign
- cardio shona sudarashan 940 766 22xx in wichita falls
- GERD (gastroesophageal reflux disease)
- Hematuria
- High cholesterol
- Myocardial infarction 2012
- one cardiac stent placed
- Ovarian mass
- Oxygen dependent
- 02 at 2LPM pulmo dr. chickenola in wichita falls
- Pneumonia
- Positive TB test
- cxr
- Seizures (HCC) 2000
- Sleep apnea
- no cpap, oxygen only

Past Surgical History:

Procedure	Laterality	Date
• HYSTERECTOMY		
• LAP-ABD DX-W/VO SPECMN-SEP PROC	N/A	12/16/2014
Procedure: LAPAROSCOPY, BILATERAL SALPINGO OOPHERECTOMY; Surgeon: Hancock, Kenneth Charles, MD; Location: THFW OR; Service: Gynecology		
• OTHER		1988
bilat breast biopsy benign		
• OTHER		2006
C-6-C7 spinal fusion		
• OTHER		1973
sx due to prolapsed uterus, cystocele & rectocele		
• OTHER		1955
frenectomy		
• OTHER		
wisdom teeth extraction		
• OTHER		
6 laser surgeries to retina of both eyes		
• OTHER		2006
bilat cataract removal		
• OTHER		
left rotator cuff repair		
• OTHER		
vag cb x 3		
• TONSILLECTOMY		

No family history on file.

Social History

Socioeconomic History

Texas Health Dallas
8200 Walnut Hill Lane

Hamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

- Marital status: Widowed
 - Spouse name: Not on file
- Number of children: Not on file
- Years of education: Not on file
- Highest education level: Not on file

Occupational History

- Not on file

Tobacco Use

- Smoking status: Former
 - Packs/day: 0.50
 - Years: 2.00
 - Additional pack years: 0.00
 - Total pack years: 1.00
 - Types: Cigarettes
- Smokeless tobacco: Not on file
- Tobacco comments:
 - Smoking History Packs/day: Tobacco use: Never smoker.

Substance and Sexual Activity

- Alcohol use: No
 - Comment: Alcoholic Drinks/day: 0
- Drug use: No
- Sexual activity: Not on file

Other Topics

- Not on file

Social History Narrative

Tobacco use: Never smoker.
No alcohol use

Social Determinants of Health

Financial Resource Strain: Not on file
Food Insecurity: Not on file
Transportation Needs: Not on file
Physical Activity: Not on file
Stress: Not on file
Social Connections: Not on file
Intimate Partner Violence: Not on file
Housing Stability: Not on file

ROS:

Review of Systems
Unable to perform ROS: Patient nonverbal

ED Triage Vitals

BP	01/27/24 1122	157/66
Pulse	01/27/24 1240	78
Resp	01/27/24 1130	17
Temp	01/27/24 1122	98.4 °F (36.9 °C)
Temp src	01/27/24 1122	Oral

Texas Health Dallas
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01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

SpO2 01/27/24 1126 96 %
Pain Level 01/27/24 1124 0
Oxygen --
Therapy Flow (L/min)
Oxygen 01/27/24 1122 room air
Therapy O2 Device
Trauma --
Activation Level
BMI 01/27/24 1122 22.83

Physical Exam:

Physical Exam

Vitals and nursing note reviewed.

Constitutional:

Appearance: She is not diaphoretic.

HENT:

Head: Normocephalic and atraumatic.

Eyes:

General:

Right eye: No discharge.

Left eye: No discharge.

Conjunctiva/sclera: Conjunctivae normal.

Neck:

Vascular: No JVD.

Trachea: No tracheal deviation.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Heart sounds: Normal heart sounds.

Pulmonary:

Effort: Pulmonary effort is normal. No respiratory distress.

Breath sounds: Normal breath sounds.

Abdominal:

General: There is no distension.

Palpations: Abdomen is soft.

Tenderness: There is no abdominal tenderness. There is no guarding or rebound.

Comments: **Temporary foley catheter in place where previous feeding tube was.**

Musculoskeletal:

General: No deformity. Normal range of motion.

Skin:

General: Skin is warm.

Capillary Refill: Capillary refill takes less than 2 seconds.

Findings: No erythema or rash.

Neurological:

Mental Status: She is alert.

Texas Health Dallas
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01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)**ED Provider Notes (continued)**

Sensory: No sensory deficit.

Comments: **Speech clear, no facial droop, MAE equally**

:

Procedures: FEEDING TUBE REPLACEMENT

Date/Time: **1/27/2024 11:28 AM**

Performed by: **Perritt, Dirk, MD**

Authorized by: **Perritt, Dirk, MD**

Consent:

Consent obtained: **Verbal**

Consent given by: **Family.**

Risks, benefits, and alternatives were discussed: **not applicable**

Risks discussed: **Bleeding, infection and pain**

Alternatives discussed: **No treatment, delayed treatment, alternative treatment, observation and referral**

Universal protocol:

Procedure explained and questions answered to patient or proxy's satisfaction: **yes**

Patient identity confirmed: **Arm band and hospital-assigned identification number**

Pre-procedure details:

Old tube type: **Gastrostomy**

Sedation:

Sedation type: **None**

Anesthesia:

Anesthesia method: **None**

Procedure details:

Patient position: **Supine**

Procedure type: **Replacement**

Tube type: **Gastrostomy**

Tube size: **18 Fr (20 Fr)**

Data**Labs:****Lab Results**

None

Radiology:**Imaging Results**

Abdomen 1 View (ABDOMEN 1 VIEW) (Final result)

Result time 01/27/24 12:29:26

Final result

Texas Health Dallas
8200 Walnut Hill Lane

Hamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024
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01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

Impression:

IMPRESSION:

1. Gastrojejunostomy tube with tip in region of proximal duodenum or gastric antrum. No extraluminal extension of contrast is visible.

2. Large amount of stool within rectum.

-Electronically Signed By: Clint Hamilton, MD

-Electronically Signed On: 1/27/2024 12:29 PM

Report Ends

Narrative:

EXAM: ABDOMEN 1 VIEW

HISTORY: Age-80 years. Gender-Female. Line Placement

TECHNIQUE: AP x-ray of the abdomen, one view.

COMPARISON: Abdomen x-ray December 26, 2023.

FINDINGS:

Lines and tubes: Contrast opacifies a gastrojejunostomy tube. The balloon appears inflated within the proximal duodenum or gastric antrum with contrast opacifying proximal duodenum. No extraluminal extension of contrast is visible.

Bowel: There is a large amount of stool within the rectum. No distended loops of bowel are present.

Soft tissues: Unremarkable.

Bones: No acute osseous abnormality.

Additional comment: None.

ED Course & Medical Decision Making

Number and Complexity of Problems

Medical Decision Making: Pt is a 80 y/o female who presents to ED for feeding tube replacement. Will replace feeding tube and order chest x-ray to ensure proper positioning.

Clinical/Quality Tools

MIPS: No pertinent MIPS measures for this encounter.

Scoring Tools:

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

Medications received in ED:

ED Medication Administration from 01/27/2024 1114 to 01/27/2024 1242

Date/Time	Order	Dose	Route	Action
01/27/2024 1200 CST	diatrizoate meg-diatrizoat sod (MD-Gastroview,Gastrografin) 66-10 % solution	25 mL	PEG TUBE	Given

ED Course as of 01/27/24 1242

Sat Jan 27, 2024

1239 Abdomen 1 View (ABDOMEN 1 VIEW)
Feeding tube in proper position. Will allow discharge.

Critical Care: None

Social Determinants of Health that impact treatment or disposition:

None

	01/27/24 1122	01/27/24 1126	01/27/24 1130	01/27/24 1240
BP:	157/66			157/66
Pulse:				78
Resp:			17	17
Temp:	98.4 °F (36.9 °C)			98.4 °F (36.9 °C)
SpO2:		96%		96%
Weight:	60.3 kg (133 lb)			
Height:	5' 4" (162.6 cm)			

Disposition

ED Disposition

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

ED Disposition Discharge Home	Condition Good	Comment --
--	-------------------	---------------

Follow-Up Information

Follow up With	Specialties	Details	Why	Contact Info
Texas Health Dallas Emergency Department Duval, Janel	Emergency Room		Immediately for any worsening as we discussed	8200 Walnut Hill Lane Dallas Texas 75231 214-345-7885 3800 N Tarrant Pkwy Ste 210 Fort Worth TX 76244- 5416 940-600-5311

There are no referrals from this visit

New Prescriptions

No medications on file

Modified Medications

No medications on file

Encounter Diagnoses

ED Diagnosis

Diagnosis	Comment	Added By	Time Added
Final diagnosis Complaint associated with gastric tube (HCC)	--	Perritt, Dirk, MD	01/27/2024 1239

Scribe: Bhavna Singh

I, Bhavna Singh, am scribing for and in the presence of Perritt, Dirk, MD.

Documentation assistance provided for Dirk Perritt, by the following scribe(s) Bhavna Singh.
Information recorded by the Scribe was done at my direction and has been reviewed and validated by me on
01/28/2024, 4:39 PM. Electronically signed by Dirk Perritt on 01/28/2024, 4:39 PM.

Electronically signed by:

Dirk A. Perritt, MD

Texas Health Dallas
8200 Walnut Hill Lane

Hamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

ED Provider Notes (continued)

Emergency Medicine Physician
Mediserv #494

Electronically signed by Perritt, Dirk, MD at 01/28/24 1639

Radiology/Imaging

Abdomen 1 View (ABDOMEN 1 VIEW) (Final result) [1418094773]

Electronically signed by: Perritt, Dirk, MD on 01/27/24 1130

Status: **Completed**

This order may be acted on in another encounter.

Ordering user: Perritt, Dirk, MD 01/27/24 1130

Authorized by: Perritt, Dirk, MD

Frequency: STAT ONCE 01/27/24 1132 - 1 occurrence

Quantity: 1

Instance released by: Perritt, Dirk, MD (auto-released) 1/27/2024 11:30 AM

Ordering provider: Perritt, Dirk, MD

Ordering mode: Standard

Class: Main Rad

Lab status: Final result

Questionnaire

Question	Answer
Reason:	Line Placement
How will the patient be transported?	Per nursing
Release to patient	Normal release

Order comments: Gastrograffin for peg confirmation

Begin Exam Questions

	Answer	Comment
Are you pregnant?		
Safe Radiology S.T.A.R. Complete?	Yes	
Does patient have an implanted or attached electronic medical device?	No	
May the exam proceed following manufacturer's recommendations?		
Physician override OR FDA Statement		

End Exam Questions

	Answer	Comment
Patient Shielded?	No	
Any Comments?		
Was this exam performed with CR technology?	No	

Abdomen 1 View (ABDOMEN 1 VIEW) [1418094773]

Resulted: 01/27/24 1229, Result status: Final result

Ordering provider: Perritt, Dirk, MD 01/27/24 1130

Resulted by: Hamilton, Clint David, MD

Performed: 01/27/24 1140 - 01/27/24 1150

Resulting lab: RAD PHP

Narrative:

EXAM: ABDOMEN 1 VIEW

Order status: Completed

Filed by: Interface, Oruradiant 01/27/24 1231

Accession number: DD168940-24

HISTORY: Age-80 years. Gender-Female. Line Placement

Texas Health Dallas
8200 Walnut Hill LaneHamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024
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01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)**Radiology/Imaging (continued)**

TECHNIQUE: AP x-ray of the abdomen, one view.

COMPARISON: Abdomen x-ray December 26, 2023.

FINDINGS:

Lines and tubes: Contrast opacifies a gastrojejunostomy tube. The balloon appears inflated within the proximal duodenum or gastric antrum with contrast opacifying proximal duodenum. No extraluminal extension of contrast is visible.

Bowel: There is a large amount of stool within the rectum. No distended loops of bowel are present.

Soft tissues: Unremarkable.

Bones: No acute osseous abnormality.

Additional comment: None.

Impression:

IMPRESSION:

1. Gastrojejunostomy tube with tip in region of proximal duodenum or gastric antrum. No extraluminal extension of contrast is visible.

2. Large amount of stool within rectum.

-Electronically Signed By: Clint Hamilton, MD

-Electronically Signed On: 1/27/2024 12:29 PM

Report Ends

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
13 - Unknown	RAD PHP	Unknown	Unknown	01/22/11 1303 - Present

Abdomen 1 View (ABDOMEN 1 VIEW) [1418094773]

Resulted: 01/27/24 1216, Result status: In process

Ordering provider: Perritt, Dirk, MD 01/27/24 1130

Order status: Completed

Resulted by: Hamilton, Clint David, MD

Filed by: Marshall, Angela, R.T. 01/27/24 1218

Performed: 01/27/24 1140 - 01/27/24 1150

Accession number: DD168940-24

Resulting lab: MISYS RAD PHD

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
11 - RADPHD	MISYS RAD PHD	Unknown	Unknown	04/13/07 1046 - Present

Signed

Electronically signed by Hamilton, Clint David, MD on 1/27/24 at 1229 CST

Results2**GLUCOSE, BEDSIDE (Final result) [1418094776]**

Electronically signed by: Interface, Orrcnerlab on 01/27/24 1451

Status: **Completed**

Ordering user: Interface, Orrcnerlab 01/27/24 1451

Authorized by: Perritt, Dirk, MD

Frequency: Routine ONCE 01/27/24 1451 - 1 occurrence

Quantity: 1

Instance released by: (auto-released) 1/27/2024 2:53 PM

Ordering provider: Perritt, Dirk, MD

Ordering mode: Standard

Class: Lab Collect

Lab status: Final result

Specimen Information

Texas Health Dallas
8200 Walnut Hill Lane

Hamlin, Sandra
MRN: 1053859, DOB: 9/26/1943, Legal Sex: F
Adm: 1/27/2024, D/C: 1/27/2024

01/27/2024 - ED in Texas Health Dallas Emergency Department (continued)

Results2 (continued)

ID	Type	Source	Collected By
1728967019	Blood	—	01/27/24 1451

GLUCOSE, BEDSIDE [1418094776] (Abnormal)

Resulted: 01/27/24 1453, Result status: Final result

Ordering provider: Perritt, Dirk, MD 01/27/24 1451

Order status: Completed

Filed by: Interface, Orrcernerlab 01/27/24 1453

Collected by: 01/27/24 1451

Resulting lab: CERNER LAB PHD

Components

Component	Value	Reference Range	Flag	Lab
Glucose, Bedside	110	65 - 100 mg/dL	H ^	PHS

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
37 - PHS	PRESBYTERIAN HOSPITAL SYSTEM	Unknown	Testing performed at facility in which patient was registered	05/19/08 1005 - Present
3307 - CERNERPHD	CERNER LAB PHD	Unknown	8200 Walnut Hill Ln DALLAS TX 75231	02/13/23 1319 - Present

Numeric Diagnostic Codes [ICD9-CM and ICD10-CM codes] found in this record are computer generated and are not intended to be used for billing purposes or as a representation of the physician's [coding] judgment.

Record Custodian's Note: In 9/14/2016, THR updated the print function of its electronic medical record to improve the formatting of the printed version of the record. Due to those formatting changes, page counts for copies of this record obtained before the update may differ from those obtained after the update. The content of the record has not changed.

Documentation found in the Fall Risk flowsheet row and marked as "visible to staff for patient care as of 10/12/22" was in development and not visible to clinicians 7/27/22-10/12/22.

end of report

Patient: SANDRA HAMLIN
Medical Record: #90008

HOPE - ADMISSION

Section A: Administrative Information

Patient / Admission Information

A0215. Site of service at admission

01. Patient's Home/Residence

A0220. Admission date

11/4/2025

A0500A. Patient first name

SANDRA

A0500B. Patient middle initial

R

A0500C. Patient last name

HAMLIN

A0550. Patient zip code

75044-3633

A0600A. Social Security Number

458-72-8838

A0600B. Patient Medicare/railroad insurance number

7TT4UP3MP93

☒ N. Patient is not a Medicaid recipient

A0810. Sex

2. Female

A0900. Birthdate

9/26/1943

A1005. Ethnicity

☒ A1005A. No, not of Hispanic, Latino/a, or Spanish origin

A1010. Race

☒ A1010A. White

Language

A1110A. Preferred Language

English

A1110B. Interpreter requested

0. No

Payor Information

☒ A1400A. Medicare (FFS)

Admitted From

A1805. Admitted From

99. Not Listed

Living Arrangements

A1905 Living Arrangements

2. With others in the home (family, friends, paid caregiver)

Availability of Assistance

A1910 Availability of Assistance

1. Around-the-clock (24 hours a day with few exceptions)

Section F: Preferences

Was patient/responsible party asked about CPR

1. Yes

F2000B. Date asked about CPR

11/4/2025

Was patient/responsible party asked about treatements other than CPR

1. Yes

F2100B. Date asked about treatment other than CPR

11/4/2025

Was patient/responsible party asked about hospitalization

1. Yes

F2200B. Date asked about hospitalization

11/4/2025

Was patient/responsible party asked about spiritual/existential concerns

1. Yes

F3000B. Date asked about spiritual/existential concerns

11/4/2025

Section I: Active Diagnoses

I0010. Principal diagnosis

04. Stroke

Comorbidities and Co-existing Conditions

- ☒ I0600. Heart Failure (e.g., congestive heart failure (CHF) and pulmonary edema)
- ☒ I0950. Cardiovascular (excluding heart failure)

Section J: Health Conditions

Pain

- J0900A. Was patient screened for pain

1. Yes
- J0900B. Date of first screening for pain

11/4/2025
- J0900C. Patient's pain severity was

1. Mild
- J0900D. Type of standardized pain tool used

9. No standardized tool used
- J0905. Is pain an active problem for the patient?

1. Yes
- J0910A. Was comprehensive pain assessment done

1. Yes
- J0910B. Date of comprehensive pain assessment

11/4/2025

Comprehensive pain assessment included:

- ☒ J0910C1. Location
- ☒ J0910C2. Severity
- ☒ J0910C3. Character
- ☒ J0910C4. Duration
- ☒ J0910C5. Frequency
- ☒ J0910C6. What relieves/worsens pain
- ☒ J0910C7. Effect on function or quality of life

J0915. Neuropathic Pain

1. Yes

Respiratory Status

- J2030A. Was patient screened for shortness of breath

1. Yes
- J2030B. Date of first screening for shortness of breath

11/4/2025
- J2030C. Did screening indicate the patient had shortness of breath

1. Yes
- J2040A. Was treatment for shortness of breath initiated

2. Yes
- J2040B. Date treatment for shortness of breath initiated

11/4/2025

Symptom Impact

- J2050A. Was a symptom impact screening completed?

1. Yes
- J2050B. Date of symptom impact screening

11/4/2025

J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	1. Slight	
B. Shortness of breath	1. Slight	
C. Anxiety	1. Slight	
D. Nausea	0. Not at all	
E. Vomiting	0. Not at all	
F. Diarrhea	0. Not at all	
G. Constipation	0. Not at all	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions? 0. No

Section N: Medications

- N0500A. Was scheduled opioid initiated or continued

0. No
- N0510A. Was PRN opioid initiated or continued

1. Yes
- N0510B. Date PRN opioid initiated or continued

11/4/2025
- N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes")

2. Yes
- N0520B. Date bowel regimen initiated or continued

11/4/2025

Section Z: Record Administration

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

- Z0500B. Date of signature verifying record completion

11/4/2025

Electronically signed by: VIRGINIA ALLAN RN ADON

Patient: SANDRA HAMLIN
Medical Record: #90008

HOPE - UPDATE VISIT

Section A: Administrative Information

A0250. Reason For Record2. HOPE Update Visit 1 (HUV1)

Patient / Admission Information

A0220. Admission date11/4/2025
A0500A. Patient first nameSANDRA
A0500B. Patient middle initialR
A0500C. Patient last nameHAMLIN
A0600A. Social Security Number458-72-8838
A0600B. Patient Medicare/railroad insurance number7TT4UP3MP93
☒ N. Patient is not a Medicaid recipient

A0810. Sex2. Female
A0900. Birthdate9/26/1943

Payor Information

☒ A1400A. Medicare (FFS)

Section J: Health Conditions

Symptom Impact

J2050A. Was a symptom impact screening completed?1. Yes
J2050B. Date of symptom impact screening11/13/2025
J2051/J2053. Symptom Impact

	J2051 Symptom Impact Screening	J2053 Symptom Impact Screening
A. Pain	0. Not at all	
B. Shortness of breath	1. Slight	
C. Anxiety	0. Not at all	
D. Nausea	0. Not at all	
E. Vomiting	0. Not at all	
F. Diarrhea	0. Not at all	
G. Constipation	0. Not at all	
H. Agitation	0. Not at all	

Section M: Skin Conditions

M1190 Does the patient have one or more skin conditions? 0. No

Section N: Medications

N0500A. Was scheduled opioid initiated or continued0. No
N0510A. Was PRN opioid initiated or continued1. Yes
N0510B. Date PRN opioid initiated or continued11/13/2025
N0520A. Was bowel regimen initiated or continued (Complete only if N0500 or N0510 is "1. Yes") 2. Yes
N0520B. Date bowel regimen initiated or continued11/13/2025

Section Z: Record Administration

Z0350. Date Assessment was Completed11/13/2025

I certify that the accompanying information accurately reflects patient assessment information for this patient and that I collected or coordinated collection of this information on the dates specified. To the best of my knowledge, this information was collected in accordance with applicable Medicare and Medicaid requirements. I understand that reporting this information is used as a basis for

payment from federal funds. I further understand that failure to report such information may lead to a 4 percentage point reduction in the Fiscal Year payment determination. I also certify that I am authorized to submit this information by this provider on its behalf.

Z0500B. Date of signature verifying record completion

11/27/2025

Electronically signed by: TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE CARE PLAN

Goals:

- ☒ Safe and effective personal care and hygiene
- ☒ Patient clean and comfortable
- ☒ Socialization/support
- ☒ Environment safe and clean

Update #1

Date: 11/4/2025 5:00 AM

Describe patient condition requiring HA interventions: Patient Requires Assistance with ADL's

Patient Info

General Condition

- ☒ Symptoms managed well

Mental/Behavioral Status

- ☒ Alert
- ☒ Confused
- ☒ Forgetful
- ☒ Resists care
- ☒ Easily agitated

Nutrition

- ☒ Diet
- Describe: As tolerated

Functional Limitations

- ☒ OOB with assist

Elimination

- ☒ Incontinent
 - ☒ Bladder
 - ☒ Bowel

Senses

- ☒ Hard of hearing
- ☒ Poor vision

Living Arrangements

☒ Caregiver present

Interventions

Pain Management

☒ Notify nurse if patient exhibits any verbal or non-verbal signs of pain or symptoms

Bathing

☒ Shower
Frequency: each visit

Hair Care

☒ Comb/brush
Frequency: each visit

Skin Care

☒ Inspect skin; check pressure areas
Frequency: each visit

☒ Call nurse if new skin problems are noted
Frequency: each visit

☒ Trim nails

☒ Fingers
Frequency: each visit

☒ Clean nails

☒ Fingers

☒ Toes
Frequency: each visit

Foot Care

☒ Bathe/inspect
Frequency: each visit

Oral Care

☒ Inspect mouth. Call nurse if problems
Frequency: each visit

☒ Assist with brushing teeth
Frequency: each visit

Elimination

- ☒ Incontinent care/diapers/briefs
- Frequency: each visit

Home Management

- ☒ Clean patient's room/area
- Frequency: each visit

Date: 11/4/2025 5:00 AM
Electronic Signature: TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/4/2025 12:45:00 PM
Date/time out: 11/4/2025 1:20:00 PM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/4/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/5/2025 10:51:00 AM
Date/time out: 11/5/2025 11:33:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/5/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/6/2025 10:50:00 AM
Date/time out: 11/6/2025 11:35:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/6/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/7/2025 10:50:00 AM
Date/time out: 11/7/2025 11:35:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/7/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/10/2025 10:50:00 AM
Date/time out: 11/10/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/10/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/11/2025 10:50:00 AM
Date/time out: 11/11/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☐ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/11/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/12/2025 10:50:00 AM
Date/time out: 11/12/2025 11:35:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/12/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/13/2025 10:50:00 AM
Date/time out: 11/13/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/13/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/14/2025 10:50:00 AM
Date/time out: 11/14/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☐ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/14/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/17/2025 10:50:00 AM
Date/time out: 11/17/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/17/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/18/2025 10:50:00 AM
Date/time out: 11/18/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/18/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/19/2025 10:50:00 AM
Date/time out: 11/19/2025 11:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/19/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/20/2025 10:50:00 AM
Date/time out: 11/20/2025 11:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☐ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/20/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/21/2025 10:50:00 AM
Date/time out: 11/21/2025 11:30:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/21/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/24/2025 10:50:00 AM
Date/time out: 11/24/2025 11:35:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/24/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/25/2025 10:50:00 AM
Date/time out: 11/25/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☐ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☐ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/25/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/26/2025 10:51:00 AM
Date/time out: 11/26/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/26/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/27/2025 10:50:00 AM
Date/time out: 11/27/2025 11:33:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/27/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 11/28/2025 10:50:00 AM
Date/time out: 11/28/2025 11:34:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 11/28/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/1/2025 10:00:00 AM
Date/time out: 12/1/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/1/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/2/2025 10:00:00 AM
Date/time out: 12/2/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☐ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/2/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/3/2025 10:02:00 AM
Date/time out: 12/3/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/3/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/5/2025 10:00:00 AM
Date/time out: 12/5/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/5/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/8/2025 10:00:00 AM
Date/time out: 12/8/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/8/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/9/2025 10:00:00 AM
Date/time out: 12/9/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/9/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/10/2025 10:00:00 AM
Date/time out: 12/10/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/10/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/11/2025 10:00:00 AM
Date/time out: 12/11/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/11/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/12/2025 10:00:00 AM
Date/time out: 12/12/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☐ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/12/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/15/2025 10:00:00 AM
Date/time out: 12/15/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/15/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

HOSPICE AIDE VISIT NOTE

HOSPICE AIDE VISIT NOTE

Date/time In: 12/16/2025 10:00:00 AM
Date/time out: 12/16/2025 10:45:00 AM

Pain Management

- ☒ No evidence of pain
- ☐ Notified nurse of patient’s pain

Bathing

- ☒ Shower

Hair Care

- ☒ Comb/brush

Skin Care

- ☒ Inspected skin; checked pressure areas
- ☒ No new skin problems
- ☐ Notified nurse of new skin problems
- ☒ Trimmed fingernails
- ☒ Cleaned/filed fingernails
- ☒ Cleaned/filed toenails

Foot Care

- ☒ Bathed and inspected feet

Oral Care

- ☒ Inspected mouth
- ☐ Notified nurse of mouth problems
- ☒ Assisted with brushing teeth

Elimination

- ☒ Provided incontinent care/changed diaper/briefs

Home Management

- ☒ Cleaned patient's room/area

Communications

Communications with nurse or supervisor (indicate date, what was reported, and to whom it was reported): None

Enter any other activities performed, and provide explanation for leaving any boxes unchecked: None
Date: 12/16/2025

Electronically signed by: SIMON ZULU CNA

Patient: SANDRA HAMLIN
Medical Record: #90008

COMFORT KIT ORDERS

All of the patient’s medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring

- ☒ Morphine Sulfate, 100mg/5ml = 20 mg/ml, 0.25-1ml, Sublingual, Q1-2 hours prn, Pain/SOB - #120mL, 00054040441
- ☒ Lorazepam Liquid, 1.75mg/ml, 0.5-1 mg, PO/SL, q4 hrs prn, Anxiety/SOB - #30mL / RF: 5, 00054353244
- ☒ Atropine sulfate oral Soluntion, 0.75%, 1-2 drops, Take Sublingually, every 2 hours as needed, for excess secretions - #15mL / RF: 11
- ☒ APAP 650MG SUPPS, 650 mg, 1 suppository, unwrap and insert Rectally, Q 4 HRS PRN, Pain/Fever - #4 / RF:11 , 00927067812
- ☒ Bisacodyl suppository, 10 mg, 1 suppository, unwrap and insert Rectally, Q DAY PRN, Constipation - #3 / RF:11 , 00363010908
- ☒ Zofran ODT, 1 tablet, 4mg, dissolve PO/SL, Q 8 HRS PRN, nausea/vomiting - #6 / RF:11
- ☒ Senna, 8.6mg, 1 Tab, PO, Daily, Constipation
- ☒ Oxygen, na, 2-4 liters, intranasally, PRN, Shortness of breath

Ordering physician: DIEGO F RESTREPO MD
Date: 11/4/2025

Electronically signed by: TRACY KING RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

DISCONTINUE MEDICATION FORM

Discontinue Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start Date: 12/3/2025
Medication: sennosides, USP 8.6 MG Oral Tablet [Senna-Time]
Unit Strength: 8.6 mg
Dose: 1
Route: po
Frequency: Bid
Indication: constipation/bowel regimen
Discontinue Date: 12/3/2025
Ordering Physician: Diago
Reviewing Physician: DIEGO F RESTREPO MD

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 12/3/2025
Ordering Physician: Dr Diego
Reviewing Physician: DIEGO F RESTREPO MD
Medication: Quetiapine
Unit strength: 50
Dose: 1
Route: Po
Frequency: At bedtime
Indication: Mood disorder
Days Supply: 30
Is medication covered? Yes

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 12/3/2025
Discontinue Date: 12/3/2025
Ordering Physician: Diego
Reviewing Physician: DIEGO F RESTREPO MD
Medication: Meatonin
Unit strength: 10
Dose: 1
Route: Po
Frequency: Bedtime
Indication: Edema
Days Supply: 30
Is medication covered? Yes

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 12/3/2025
Ordering Physician: Diego
Reviewing Physician: DIEGO F RESTREPO MD
Medication: Furosemide
Unit strength: 20
Dose: 1
Route: By mouth
Frequency: Twice a day
Indication: Edema
Days Supply: 30
Is medication covered? Yes

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 12/3/2025
Ordering Physician: Diego
Reviewing Physician: DIEGO F RESTREPO MD
Medication: Potassium
Unit strength: 10
Dose: 1
Route: Po
Frequency: Daily
Indication: Supplement
Days Supply: 30
Is medication covered? Yes

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

MEDICATION FORM

Medication Form

All of the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Start date: 12/3/2025
Ordering Physician: Diego
Reviewing Physician: DIEGO F RESTREPO MD
Medication: Melatonin
Unit strength: 10
Dose: 1
Route: By mouth
Frequency: Qhs
Indication: Insomnia
Days Supply: 30
Is medication covered? Yes

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

COVID-19 ASSESSMENT

COVID-19 ASSESSMENT

This assessment tool supports you in identifying patients at risk for COVID-19 and helps guide clinicians in clinical decision making.

Symptoms

Note: Symptoms may appear 2-14 days after exposure.

☒ None

Travel History

***Updated information on affected countries can be found at: <https://www.cdc.gov/coronavirus/2019-ncov/travelers/index.html>*

Action Taken

☒ None indicated

***Please refer to the CDC COVID-19 website for ongoing updates and guidance: <https://www.cdc.gov/coronavirus/2019-ncov>*

COVID-19 TESTING

Patient testing completed? No

Date: 11/4/2025

Electronically signed by: TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In: 11/4/2025 3:45:00 PM
Date/Time Out: 11/4/2025 4:15:00 PM
Type of visit: PRN
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No
Narrative: UPON VISIT PATIENT NOTED LYING IN BED AWAKE IN PLEASANT MOOD, DENIES PAIN OR DISCOMFORT IN STABLE CONDITION, V/S T96.7 P71 R18 B/P 123/57 BREATH SOUNDS CLEAR SPO2 96 % RA. NO COMPLAINTS WILL CONTINUE CURRENT PLAN OF CARE.
Last bowel movement: 11/4/2025
Pain: 0
PRN medication usage last 24 hours: 0
Need medication refills: No
Discussion with caregiver/POA/facility staff: Discussed with caregiver to monitor oxygen level more frequent
Date: 11/4/2025

Electronically signed by: DOROTHY JONES LVN

Patient: SANDRA HAMLIN
Medical Record: #90008

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:11/10/2025 11:30:00 AM

Date/Time out:11/10/2025 12:30:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during visit:none

Patient/Family/Caregiver's primary concern during this visit:none

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 97.6
- Respiratory Rate

Respiratory rate (respirations per minute): 16
- Heart Rate

Type of heart rate: Regular

Radial pulse: 70
- Blood Pressure

Hypertension or hypotension? Hypertension controlled with medication

☒ Sitting

Systolic blood pressure: 125

Diastolic blood pressure: 67
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented
- ☒ Disoriented
- ☒ Confused

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative
- ☒ Withdrawn

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: generalized

Neurosensory

- ☒ Dizziness

Cardiovascular

- ☒ Chest pain
- ☒ Capillary refill > 3 seconds
- ☒ Edema

Location: bilateral lower extremities
Rating: +2

Nutrition

- ☒ Patient satisfied with intake
- Percent of meals eaten: 50
- ☒ Patent
 - ☒ Intact

GI Elimination

Date of last bowel movement: 11/10/2025
Bowel Pattern daily
Bowel sounds, # of quads: 4

- ☒ WNL

Describe: times 4

- ☒ Abdomen: Soft
- ☒ Incontinence

GU Elimination

Urine output: Fair

- ☒ Frequency

Describe: clear

- ☒ Clear
- ☒ Incontinent

Respiratory

☒ Diminished

Describe location, inspiratory/expiratory: lower extremities

☒ Dyspnea at rest

☒ Dyspnea with exertion

Mobility

☒ Ambulatory

Steady/unsteady gait: Steady gait

☒ Poor endurance

☒ Weakness/fatigue

Skin

Skin temperature: warm

Skin turgor: poor

Psychosocial Status

Psychosocial/emotional status: stable

Anticipatory grief/bereavement status: normal

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide

Hospice caregiver name: DAVIS, DEMETRICE

Present/not present Not present

• Interview With

☒ PCG

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

• Patient/family/CG satisfied with care provided

☒ Yes

• Treats patient with dignity and respect

☒ Yes

• Interacts well with patient/family/CG

☒ Yes

• Provides care in a safe manner

☒ Yes

• Arrives on time or calls if late

☒ Yes

- *Competent with assigned tasks*
☒ Yes
- *Complies with infection control procedures*
☒ Yes
- *Reports changes in patient's condition to RN*
☒ Yes
- *Patient appears well groomed*
☒ Yes
- *Patient's nails are clean and trimmed*
☒ Yes
- *Patient's hair is cleaned*
☒ Yes
- *Patient's mouth is clean and odor-free*
☒ Yes
- *Patient is clean-shaven*
☒ N/A
- *Patient's skin is moisturized*
☒ Yes
- *Patient's room is clean and tidy*
☒ Yes
- *Patient's linens are fresh and clean*
☒ Yes
- *Assignment sheet/POC reviewed*
☒ Yes
- *Assignment sheet/POC updated*
☒ N/A

RN Supervisory Visit (2)

Hospice Caregiver Discipline	LPN
Hospice caregiver name:	Taneka Davis
Present/not present:	Not present

- *Interview With*
☒ PCG
- *Follows assignment/plan of care*
☒ Yes

- *Patient's needs are being met*
☒ Yes
- *Patient/family/CG satisfied with care provided*
☒ Yes
- *Treats patient with dignity and respect*
☒ Yes
- *Complies with infection control procedures*
☒ Yes
- *Reports changes in patient's condition to RN*
☒ Yes
- *Punctual: notifies office if late*
☒ Yes
- *Dresses properly*
☒ Yes
- *Interacts well with patient/family/CG*
☒ Yes
- *Provides care in a safe manner*
☒ Yes
- *Arrives on time or calls if late*
☒ Yes
- *Competent with assigned tasks*
☒ Yes

HOPE Update Visit

Reason for Visit	2. HOPE Update Visit 1 (HUV1)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes
Date of symptom impact screening	11/10/2025
Pain	1. Slight
Shortness of breath	1. Slight
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	1. Slight
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No
Was PRN opioid initiated or continued? 1. Yes
Date PRN opioid initiated or continued 11/10/2025
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 11/10/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):
Hospice philosophy and goals of care
Roles of the hospice team (RN, MD, CNA, social worker, chaplain)
Plan of care, visit frequency, and 24/7 on-call availability
Symptom management (pain, shortness of breath, nausea, anxiety, constipation)
Use, purpose, and possible side effects of current medications and comfort kit
Safety and fall precautions in the home/facility
When to call hospice vs. 911
Patient and family rights, responsibilities, and informed consent
Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Communication with family: Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.
☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Date: 11/10/2025

Electronically signed by: TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:11/13/2025 11:40:00 AM

Date/Time out:11/13/2025 12:40:00 PM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Vital Signs

- Temperature
- Type of temperature taken: Temporal:
Temporal temperature: 97
- Respiratory Rate
- Respiratory rate (respirations per minute): 18
Oxygen liters per minute: 97
- Heart Rate
- Type of heart rate: Regular
Radial pulse: 68
- Blood Pressure
- ☒ Laying
Systolic blood pressure: 122
Diastolic blood pressure: 67
- Pedal Pulses
- ☒ Present

Current Mental Status

- ☒ Alert
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear

Pain Assessment

Pain rating type: Verbal Scale

Pain Rating: 0

Nutrition

☒ Patient satisfied with intake

Percent of meals eaten: 50

GI Elimination

Date of last bowel movement: 11/13/2025

Bowel Pattern Regular

Bowel sounds, # of quads: 4

☒ WNL

☒ Abdomen: Soft

☒ Incontinence

GU Elimination

☒ Incontinent

Respiratory

☒ Clear

Describe location, inspiratory/expiratory: Bil

Mobility

☒ Bed bound

☒ Up to chair/wheelchair

Skin

Skin temperature: Warm

Skin turgor: Poor

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide

Hospice caregiver name: Simon

Present/not present Not present

• Interview With

☒ Family

☒ PCG

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

• Patient/family/CG satisfied with care provided

☒ Yes

• Treats patient with dignity and respect

☒ Yes

- *Interacts well with patient/family/CG*

☒ Yes

- *Provides care in a safe manner*

☒ Yes

- *Arrives on time or calls if late*

☒ Yes

- *Competent with assigned tasks*

☒ Yes

- *Complies with infection control procedures*

☒ Yes

- *Reports changes in patient's condition to RN*

☒ Yes

- *Patient appears well groomed*

☒ Yes

- *Patient's nails are clean and trimmed*

☒ Yes

- *Patient's hair is cleaned*

☒ Yes

- *Patient's mouth is clean and odor-free*

☒ Yes

- *Patient is clean-shaven*

☒ Yes

- *Patient's skin is moisturized*

☒ Yes

- *Patient's room is clean and tidy*

☒ Yes

- *Patient's linens are fresh and clean*

☒ Yes

- *Assignment sheet/POC reviewed*

☒ Yes

- *Assignment sheet/POC updated*

☒ Yes

HOPE Update Visit

Reason for Visit2. HOPE Update Visit 1 (HUV1)

At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?0. No

Was a symptom impact screening completed?1. Yes

Date of symptom impact screening11/13/2025

Pain0. Not at all

Shortness of breath1. Slight

Anxiety0. Not at all

Nausea0. Not at all

Vomiting0. Not at all

Diarrhea0. Not at all

Constipation0. Not at all

Agitation0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued?0. No

Was PRN opioid initiated or continued?1. Yes

Date PRN opioid initiated or continued11/13/2025

Was bowel regimen initiated or continued?2. Yes

Date bowel regimen initiated or continued11/13/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? No

Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):

Patient receives sitting in her wheelchair. Patient is weak looks very tired, respiration even and unable. Patient was kind of sleeping in and out during assessment no s/ and symptoms of pain noted and Caregiver denies patient having any pain. Symptoms are managed

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

☒ Caregiver reminded to call hospice and not 911 for any concerns, problems or changes in condition

Date: 11/13/2025

Electronically signed by: STELLA AHANOTU RN

Patient: SANDRA HAMLIN
Medical Record: #90008

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In: 11/18/2025 2:00:00 PM
Date/Time Out: 11/18/2025 2:30:00 PM
Type of visit: PRN
COVID-19 screening performed? Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Narrative: Upon visit patient noted lying in bed resting alert x 1-2 patient is non verbal. Patient did not appear to be in pain no facial grimace or moaning present . V/S T97.6 P 70 R 28 B/P 125/63 SPO2 95 % RA. No SOB noted HOB raised . Caregiver stated patient has been stable this week no complaints. Will continue current plan of care

Last bowel movement: 11/18/2025

Pain: 0

PRN medication usage last 24 hours: 0

Need medication refills: Yes

Discussion with caregiver/POA/facility staff: Discussed with caregiver to check O2 Sats daily.

Date: 11/18/2025

Electronically signed by: DOROTHY JONES LVN

Patient: SANDRA HAMLIN
Medical Record: #90008

HUV NURSING VISIT NOTE

NURSE COMPREHENSIVE VISIT

Date/Time in:11/20/2025 10:00:00 AM

Date/Time out:11/20/2025 11:00:00 AM

COVID-19 screening performed?Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Others present during visit:none

Patient/Family/Caregiver's primary concern during this visit:none

Vital Signs

- Temperature

Type of temperature taken: Temporal:

Temporal temperature: 97.7
- Respiratory Rate

Respiratory rate (respirations per minute): 16

☒ Room air
- Heart Rate

Type of heart rate: Regular

Radial pulse: 72
- Blood Pressure

Hypertension or hypotension? Hypertension controlled with medication

☒ Sitting

Systolic blood pressure: 136

Diastolic blood pressure: 78
- Pedal Pulses

☒ Present

☒ Right

☒ Left

Current Mental Status

- ☒ Alert
- ☒ Oriented
- ☒ Confused
- ☒ Forgetful

Current Behavioral Status

- ☒ Appropriate
- ☒ Cooperative

Speech Status

- ☒ Clear
- ☒ Appropriate

Pain Assessment

Pain rating type: Verbal Scale
Pain Rating: 1
New, controlled, or uncontrolled? Controlled
Location/description of pain: genralized

Neurosensory

- ☒ Dizziness

Cardiovascular

- ☒ Syncope
- ☒ Capillary refill > 3 seconds
- ☒ Edema

Location: bilateral lower extremities
Rating: +2

Nutrition

- ☒ Patient satisfied with intake
- Percent of meals eaten: 50
- ☒ Patent
 - ☒ Intact

GI Elimination

Date of last bowel movement: 11/20/2025
Bowel Pattern daily
Bowel sounds, # of quads: 4

- ☒ WNL

Describe: times 4

- ☒ Abdomen: Soft
- ☒ Incontinence

GU Elimination

Urine output: Fair

- ☒ Frequency

Describe: clear

- ☒ Clear
- ☒ Incontinent

Respiratory

☒ Diminished

Describe location, inspiratory/expiratory: lower extremities

☒ Dyspnea at rest

☒ Dyspnea with exertion

Mobility

☒ Ambulatory

Steady/unsteady gait: Steady gait

☒ Poor endurance

☒ Weakness/fatigue

Skin

Skin temperature: warm

Skin turgor: poor

Psychosocial Status

Psychosocial/emotional status: stable

Anticipatory grief/bereavement status: normal

RN Supervisory Visit (1)

Hospice Caregiver Discipline Hospice Aide

Hospice caregiver name: DAVIS, DEMETRICE

Present/not present Not present

• Interview With

☒ PCG

• Follows assignment/plan of care

☒ Yes

• Patient's needs are being met

☒ Yes

• Patient/family/CG satisfied with care provided

☒ Yes

• Treats patient with dignity and respect

☒ Yes

• Interacts well with patient/family/CG

☒ Yes

• Provides care in a safe manner

☒ Yes

• Arrives on time or calls if late

☒ Yes

- *Competent with assigned tasks*
☒ Yes
- *Complies with infection control procedures*
☒ Yes
- *Reports changes in patient's condition to RN*
☒ Yes
- *Patient appears well groomed*
☒ Yes
- *Patient's nails are clean and trimmed*
☒ Yes
- *Patient's hair is cleaned*
☒ Yes
- *Patient's mouth is clean and odor-free*
☒ Yes
- *Patient is clean-shaven*
☒ N/A
- *Patient's skin is moisturized*
☒ Yes
- *Patient's room is clean and tidy*
☒ Yes
- *Patient's linens are fresh and clean*
☒ Yes
- *Assignment sheet/POC reviewed*
☒ Yes
- *Assignment sheet/POC updated*
☒ N/A

RN Supervisory Visit (2)

Hospice Caregiver Discipline	LPN
Present/not present:	Not present

- *Interview With*
☒ PCG
- *Follows assignment/plan of care*
☒ Yes

- *Patient's needs are being met*
☒ Yes
- *Patient/family/CG satisfied with care provided*
☒ Yes
- *Treats patient with dignity and respect*
☒ Yes
- *Complies with infection control procedures*
☒ Yes
- *Reports changes in patient's condition to RN*
☒ Yes
- *Punctual: notifies office if late*
☒ Yes
- *Dresses properly*
☒ Yes
- *Interacts well with patient/family/CG*
☒ Yes
- *Provides care in a safe manner*
☒ Yes
- *Arrives on time or calls if late*
☒ Yes
- *Competent with assigned tasks*
☒ Yes

HOPE Update Visit

Reason for Visit	3. HOPE Update Visit 2 (HUV2)
At the time of this assessment and based on your clinical assessment, does the patient appear to have a life expectancy of 3 days or less?	0. No
Was a symptom impact screening completed?	1. Yes
Date of symptom impact screening	11/20/2025
Pain	1. Slight
Shortness of breath	1. Slight
Anxiety	1. Slight
Nausea	0. Not at all
Vomiting	0. Not at all
Diarrhea	0. Not at all
Constipation	1. Slight
Agitation	0. Not at all

Skin Conditions

Does the patient have one or more skin conditions? 0. No

Medications

Was scheduled opioid initiated or continued? 0. No
Was PRN opioid initiated or continued? 1. Yes
Date PRN opioid initiated or continued 11/20/2025
Was bowel regimen initiated or continued? 2. Yes
Date bowel regimen initiated or continued 11/20/2025

Medication Utilization

Have any PRN medications been used in the last 24 hours? Yes
Does patient require refill of any medication? No

Communication

• Teaching provided during visit

Topic(s) discussed (include topic(s) and person(s) receiving instruction):
Hospice philosophy and goals of care
Roles of the hospice team (RN, MD, CNA, social worker, chaplain)
Plan of care, visit frequency, and 24/7 on-call availability
Symptom management (pain, shortness of breath, nausea, anxiety, constipation)
Use, purpose, and possible side effects of current medications and comfort kit
Safety and fall precautions in the home/facility
When to call hospice vs. 911
Patient and family rights, responsibilities, and informed consent
Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.

• Method(s)

☒ Verbal instruction

• Outcome(s)

☒ Verbalized understanding

Communication with family: Patient and family verbalized understanding, asked appropriate questions, and agreed with the plan of care.
☒ Patient reminded to call hospice and not 911 for any concerns, problems or changes in condition
☒ Caregiver/Family educated about HCAHPS survey process as appropriate

Date: 11/20/2025

Electronically signed by: TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In: 12/2/2025 12:00:00 PM
Date/Time Out: 12/2/2025 12:30:00 PM
Type of visit: Scheduled
COVID-19 screening performed? Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No

Narrative: upon visit patient lying in bed resting, alert x2-3, able to voice needs denies pain or discomfort, caregiver stated patient has been in stable condition no issues noted . v/s t97.2 p69 R18 b/p 117/64 spo2 96% ra. no sob noted , patient in pleasant mood will continue current plan of care.

Last bowel movement: 12/2/2025

Pain: 0
PRN medication usage last 24 hours: 0
Need medication refills: No

Discussion with caregiver/POA/facility staff: discussed with caregiver to monitor pain levels and give pain medications as ordered for shoulder pain patient has oftgen

Date: 12/2/2025

Electronically signed by: DOROTHY JONES LVN

Patient: SANDRA HAMLIN
Medical Record: #90008

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In: 12/12/2025 8:00:00 AM
Date/Time Out: 12/12/2025 8:30:00 AM
Type of visit: Scheduled
COVID-19 screening performed? Yes
Is the patient and/or anyone living in the home displaying any symptoms of COVID-19? No
Narrative: upon visit patient noted sitting in chair alert , breath sounds clear ,skin warm and dry, abdomen soft not distended , patient is non verbal, v/s 97.1 p64 R18 b/p 127/60 spo2 96 % ra, caregiver stated patient needed refill of medication.
Last bowel movement: 12/12/2025
Pain: 0
PRN medication usage last 24 hours: 0
Need medication refills: Yes
Discussion with caregiver/POA/facility staff: discussed with caregiver to enforce fluids to prevent dehydration.
Date: 12/12/2025

Electronically signed by: DOROTHY JONES LVN

Patient: SANDRA HAMLIN
Medical Record: #90008

NURSING VISIT NOTE (Abridged)

NURSING VISIT NOTE (Abridged)

Date/Time In:

12/16/2025 1:00:00 PM

Date/Time Out:

12/16/2025 1:30:00 PM

Type of visit:

Scheduled

COVID-19 screening performed?

Yes

Is the patient and/or anyone living in the home displaying any symptoms of COVID-19?

No

Narrative:

upon visit patient noted lying in bed resting , hob raised breath sounds clear no sob noted , skin clear turgor sluggish, v/s within normal range last bowel movement today sized large. no complaints today. caregiver stated patient has been in pleasant mood today no issues or concerns noted.

Last bowel movement:

12/16/2025

Pain:

0

PRN medication usage last 24 hours:

0

Need medication refills:

No

Discussion with caregiver/POA/facility staff:

discussed with caregiver the importance of turning patient q 2 hrs while in bed to prevent pressure ulcers.

Date:

12/16/2025

Electronically signed by: DOROTHY JONES LVN

Patient: SANDRA HAMLIN
Medical Record: #90008

TREATMENT ORDERS

TREATMENT ORDERS

Ordering physician: DIEGO F RESTREPO MD
Order: Admit to Hospice: Dx: Cerebral Infarction, unspecified.
Patient is an OOH-DNR
Diet & Activity as tolerated
DME: Hospital Bed, Bedside Table, BSC, Wheelchair
Date ordered: 11/4/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

TREATMENT ORDERS

TREATMENT ORDERS

Ordering physician: DIEGO F RESTREPO MD
Order: Trazodone 150mg. Take 1 tablet at bedtime for insomnia
Tylenol 500mg . Take 1 tablet by mouth q6hrs prn for pain
Date ordered: 11/20/2025

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

ADMISSION ORDERS / INITIAL PLAN OF CARE

Skilled Nursing Orders

- ☒ Admit to hospice.
- Level of care: Routine Home Care
- ☒ Assess vital signs and physiological systems as needed.
- ☒ Assess pain and symptoms and need for further intervention.
- ☒ Teach need for giving pain medications around-the-clock and potential side effects of medications.
- ☒ May give medications related to palliative care sublingually or rectally if patient is unable to swallow.
- ☒ May anchor Foley catheter and change/irrigate per hospice protocol.
- ☒ Assess bowel status with each visit.
- ☒ May check for impaction PRN.
- ☒ Oxygen at 2-4 L per nasal cannula PRN for dyspnea and/or respiratory distress.
- ☒ May check pulse oximetry for dyspnea, altered respirations, altered mental status and prior to ordering oxygen. Notify physician if reading is under 90%.
- ☒ Initiate measures to prevent/manage skin breakdown: hydrocolloid dressing, pressure-relieving mattress PRN.
- ☒ Teach and support caregiver in care of patient.
- ☒ Teach safety measures, infection control, and emergency preparedness.
- ☒ Medication orders attached.
- ☒ May place Hospice Emergency Medication Kit in patient's residence and use PRN as directed.

Nutrition Orders

- ☒ Oral diet orders
- Describe: Diet as tolerated

Activity Orders

- ☒ As tolerated

Durable Medical Equipment Orders

- ☒ Hospital bed
- ☒ Overbed table
- ☒ Oxygen

INITIAL PLAN OF CARE

Skilled Nursing Visits

- ☒ Supervisory Visits every 2 weeks if patient receives Hospice Aide services
- Weekly Visit Frequency (to be reassessed at next IDG): 1
- +PRN Weekly Visits 1
- Reason for PRN Visits change in condition

Social Worker Visits

- ☒ Psychosocial/Anticipatory Grief/Bereavement assessment
- ☒ Assist patient/family in planning future care
- ☒ Assist with access to community resources
- ☒ Assist with Advance Directives
- ☒ Assist with funeral planning
- ☒ Assist patient/family in obtaining financial assistance
- ☒ Educate patient/family in coping with loss/disease, grief symptoms, anticipatory grief, and available hospice services
- ☒ Evaluation for volunteer services

Visit Frequency for next month (to be reassessed at next IDG): 1
+PRN Visits for next month: 1

Spiritual Visits

- ☒ Spiritual needs assessment
- ☒ Phone contacts and/or visits to patient/family
- ☒ Phone call to patient/family's own pastor
- ☒ Patient/family receive visits from their own pastor

Visit Frequency for next month (to be reassessed at next IDG): 1
+PRN Visits for next month: 1

Hospice Aide Visits

- ☒ Assist with personal care/hygiene and ADLs
- ☒ Light housekeeping
- ☒ Aide Assignment Sheet/Plan of Care completed

Weekly Visit Frequency (to be reassessed at next IDG): 5

Therapy Visits

- ☒ No therapy needs identified at this time

Volunteer Visits

- ☒ Volunteer services were explained to patient/POA.

Medication Review

- ☒ Medical Director and/or Attending Physician has reviewed all the medications including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.
- ☒ All medications have been reviewed and reconciled with Attending Physician and/or Medical Director.

As the Interdisciplinary Team members, we have participated in the planning of this patient's care prior to providing care (as applicable)

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Clinical Director: VIRGINIA ALLAN RN ADON
Registered Nurse: STELLA AHANOTU RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor: DAMIEN DAWSON M.DIV
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 11/4/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD
Electronically signed by: ASHLEY DAVIS LMSW
Electronically signed by: VERNON MORRISON
Electronically signed by: STELLA AHANOTU RN
Electronically signed by: TRACY KING RN
Electronically signed by: DAMIEN DAWSON M.DIV

Patient: SANDRA HAMLIN
Medical Record: #90008

ADMISSION ORDERS / INITIAL PLAN OF CARE

Skilled Nursing Orders

- ☒ Admit to hospice.
- Level of care: Routine Home Care
- ☒ Assess vital signs and physiological systems as needed.
- ☒ Assess pain and symptoms and need for further intervention.
- ☒ Teach need for giving pain medications around-the-clock and potential side effects of medications.
- ☒ May give medications related to palliative care sublingually or rectally if patient is unable to swallow.
- ☒ May anchor Foley catheter and change/irrigate per hospice protocol.
- ☒ Assess bowel status with each visit.
- ☒ May check for impaction PRN.
- ☒ Oxygen at 2-4 L per nasal cannula PRN for dyspnea and/or respiratory distress.
- ☒ May check pulse oximetry for dyspnea, altered respirations, altered mental status and prior to ordering oxygen. Notify physician if reading is under 90%.
- ☒ Initiate measures to prevent/manage skin breakdown: hydrocolloid dressing, pressure-relieving mattress PRN.
- ☒ Teach and support caregiver in care of patient.
- ☒ Teach safety measures, infection control, and emergency preparedness.
- ☒ Medication orders attached.
- ☒ May place Hospice Emergency Medication Kit in patient's residence and use PRN as directed.

Nutrition Orders

- ☒ Oral diet orders
- Describe: diet as tolerated

Activity Orders

- ☒ As tolerated

Durable Medical Equipment Orders

- ☒ Hospital bed
- ☒ Overbed table
- ☒ Oxygen

INITIAL PLAN OF CARE

Skilled Nursing Visits

- ☒ Supervisory Visits every 2 weeks if patient receives Hospice Aide services
- Weekly Visit Frequency (to be reassessed at next IDG): 1
- +PRN Weekly Visits 1
- Reason for PRN Visits change in condition

Social Worker Visits

- ☒ Psychosocial/Anticipatory Grief/Bereavement assessment
 - ☒ Assist patient/family in planning future care
 - ☒ Assist with access to community resources
 - ☒ Assist with Advance Directives
 - ☒ Assist with funeral planning
 - ☒ Assist patient/family in obtaining financial assistance
 - ☒ Educate patient/family in coping with loss/disease, grief symptoms, anticipatory grief, and available hospice services
 - ☒ Evaluation for volunteer services
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Spiritual Visits

- ☒ Spiritual needs assessment
 - ☒ Phone call to patient/family's own pastor
 - ☒ Patient/family receive visits from their own pastor
- Visit Frequency for next month (to be reassessed at next IDG): 1
- +PRN Visits for next month: 1

Hospice Aide Visits

- ☒ Assist with personal care/hygiene and ADLs
 - ☒ Light housekeeping
 - ☒ Aide Assignment Sheet/Plan of Care completed
- Weekly Visit Frequency (to be reassessed at next IDG): 5

Therapy Visits

- ☒ No therapy needs identified at this time

Volunteer Visits

- ☒ Volunteer services were explained to patient/POA.

Medication Review

- ☒ Medical Director and/or Attending Physician has reviewed all the medications including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.
- ☒ All medications have been reviewed and reconciled with Attending Physician and/or Medical Director.

As the Interdisciplinary Team members, we have participated in the planning of this patient's care prior to providing care (as applicable)

Medical Director/Physician Designee: DIEGO F RESTREPO MD

Clinical Director: TRACY KING RN

Registered Nurse: VIRGINIA ALLAN RN ADON

Social Worker: ASHLEY DAVIS LMSW

Spiritual Counselor: VERNON MORRISON

Volunteer Coordinator: VERNON MORRISON

Bereavement Coordinator: VERNON MORRISON

Date: 11/4/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: TRACY KING RN
Electronically signed by: VERNON MORRISON
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

IDG UPDATE

From date: 11/6/2025
To date: 11/20/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update: All equipment was provided at admission

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:	Start Date	DC Date	Medication	Unit	Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
	Regular Medications:										
	No medications										
	Comfort Kit Medications:										
	No medications										

Medication list reflects medication orders as of 11/6/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:	Problem	Start Date	Latest Update
	No Care Plans		

Care Plan list reflects care plan orders as of 11/6/2025

Treatment	Name	Start Date	DC Date

Order List:

No Physician Orders

Physician orders list reflects physician orders as of 11/6/2025

IDG Narrative: Sandra Hamlin, an 82 -year-old widowed female with a primary diagnosis of cerebral infarction, unspecified (I63.9), is currently under routine hospice services. She presents with related conditions including unspecified pain (R52), nausea with vomiting (R11.2), gastro-esophageal reflux disease with esophagitis, without bleeding (K21.00), and unspecified protein-calorie malnutrition (E46). Ms. Hamlin's current healthcare needs are being managed under the supervision of Hospice Physician Dr. Diego Restrepo.

Ms. Hamlin has a history of multiple hospital admissions for suspected cancer, CVA, MI, syncope, history of GT placement. She has right-sided weakness, severe hypertension, and an episode of syncope. Despite being sent home with private in-home care, Ms. Hamlin's condition has continued to decline, evidenced by a progressive weight loss of 11 pounds over the last two months, bringing her weight to 130 pounds and a BMI of 21. Her serum albumin level is notably low at 2.5 g/dL, indicating impaired nutritional status. Her Karnofsky Performance Status is 50%, and the Palliative Performance Scale (PPS) is also 50%, signifying moderate disease progression with significant care needs.

Physical assessment denotes the following:

- Height: 5'4"
- Mid-arm circumference (LMAC): 22 cm
- Activities of Daily Living (ADLs): 6/6, requiring total care in all areas.

Ms. Hamlin reports a general increase in lethargy, sleeping approximately 14 hours daily. Her current pain level, assessed using the Pain Assessment in Advanced Dementia (PAINAD) scale, is 2, suggesting mild pain.

Ms. Hamlin's care plan focuses on symptom management and comfort. Interventions include regular monitoring of her nutritional intake and hydration status, management of pain and other related symptoms, and providing ongoing emotional and spiritual support in accordance with her Christian faith. Discussions with her caregiver support the ongoing need for guidance and resources to manage her evolving care needs effectively. As per her wishes, she holds a Do Not Resuscitate (DNR) order. The patient is no longer seeking aggressive treatment.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: JAMES KIZITO RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor DAMIEN DAWSON M.DIV
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 11/6/2025

Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DAMIEN DAWSON M.DIV
Electronically signed by: VERNON MORRISON
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

CARE PLAN

Problem being care planned: Anxiety/Agitation

Measurable Goals: Family/caregivers will verbalize understanding of terminal restlessness and its management

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Anxiety/Agitation	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind Instruct family/caregiver on causes of terminal restlessness and what to expect.	11/7/2025 3:53 AM	TRACY KING RN
2	Anxiety/Agitation	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind Instruct family/caregiver on causes of terminal restlessness and what to expect.	11/20/2025 1:59 AM	TRACY KING RN
3	Anxiety/Agitation	1	Assess for possible psychosocial or spiritual causes of anxiety: Depression, Fear of pain, Fear of the dying process, Spiritual distress, Fear of life after death, Lack of earthly reconciliation or forgiveness, Unfinished business, Concerns about family left behind Instruct family/caregiver on causes of terminal restlessness and what to expect.	12/3/2025 10:55 PM	TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

CARE PLAN

Problem being care planned: Constipation
Measurable Goals: Patient will report no discomfort with elimination.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	potential for Constipation	1	Assess for possible causes of constipation: Ovarian cancer, Stomach, colon or rectal cancer, Prostate or bladder cancer, Lymphoma, Immobility, Lack of fiber in diet, Dehydration or decreased fluid intake, Laxative abuse, Autonomic neuropathy, Hypercalcemia, Paralysis or spinal cord injury, Confusion or inattention, Hypothyroid, Uremia, Parkinson's Disease, Multiple sclerosis, Dementia, Stress or lack of privacy, Depression, Medication side effects Instruct patient and family/caregiver on disease process, bowel management, diet, exercise, fluid intake.	11/7/2025 3:54 AM	TRACY KING RN
2	potential for Constipation	1	Assess for possible causes of constipation: Ovarian cancer, Stomach, colon or rectal cancer, Prostate or bladder cancer, Lymphoma, Immobility, Lack of fiber in diet, Dehydration or decreased fluid intake, Laxative abuse, Autonomic neuropathy, Hypercalcemia, Paralysis or spinal cord injury, Confusion or inattention, Hypothyroid, Uremia, Parkinson's Disease, Multiple sclerosis, Dementia, Stress or lack of privacy, Depression, Medication side effects Instruct patient and family/caregiver on disease process, bowel management, diet, exercise, fluid intake.	11/20/2025 1:58 AM	TRACY KING RN
3	potential for Constipation	1	Assess for possible causes of constipation: Ovarian cancer, Stomach, colon or rectal cancer, Prostate or bladder cancer, Lymphoma, Immobility, Lack of fiber in diet, Dehydration or decreased fluid intake, Laxative abuse, Autonomic neuropathy, Hypercalcemia, Paralysis or spinal cord injury, Confusion or inattention, Hypothyroid, Uremia, Parkinson's Disease, Multiple sclerosis, Dementia, Stress or lack of privacy, Depression, Medication side effects Instruct patient and family/caregiver on disease process, bowel management, diet, exercise, fluid intake.	12/3/2025 10:54 PM	TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

CARE PLAN

Problem being care planned: Dyspnea
Measurable Goals: Patient will report decreased feelings of breathlessness and increased comfort.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	potential for Constipation	2	If patient reports improved symptoms with use of oxygen, continue oxygen therapy. Instruct patient and family/caregiver on relaxation techniques:, Guided imagery, Therapeutic touch, Music Provide Patient Education Handout(s):, Food Sources of Magnesium, Low Blood Magnesium, Paracentesis, Removal of a Breathing Machine, Anemia-Related Fatigue, About Your Oxygen, BiPAP and CPAP, Breathing Exercises, CPAP, Dry Powder Inhaler (DPI), How to Count Pulse and Respirations, Humidifiers and Vaporizers, Laryngectomy Suctioning, Metered Dose Inhaler (MDI), Nebulizer Treatments, Oxygen - Cleaning and Maintaining System, Oxygen Concentrator, Oxygen Concentrator - Manufacturer's Operating Instructions, Oxygen Safety, Pulse Oximeter, Pursed Lip Breathing, Tracheostomy - Deep Breathing and Coughing, Tracheostomy Suctioning, Tracheostomy Tubes, Tracheostomy-Laryngectomy - When to Call Hospice, Tuberculosis, Using a Spacer with Your Inhaler, When You're Feeling Short of Breath	11/7/2025 3:56 AM	TRACY KING RN
2	potential for Constipation	2	If patient reports improved symptoms with use of oxygen, continue oxygen therapy. Instruct patient and family/caregiver on relaxation techniques:, Guided imagery, Therapeutic touch, Music Provide Patient Education Handout(s):, Food Sources of Magnesium, Low Blood Magnesium, Paracentesis, Removal of a Breathing Machine, Anemia-Related Fatigue, About Your Oxygen, BiPAP and CPAP, Breathing Exercises, CPAP, Dry Powder Inhaler (DPI), How to Count Pulse and Respirations, Humidifiers and Vaporizers, Laryngectomy Suctioning, Metered Dose Inhaler (MDI), Nebulizer Treatments, Oxygen - Cleaning and Maintaining System, Oxygen Concentrator, Oxygen Concentrator - Manufacturer's Operating Instructions, Oxygen Safety, Pulse Oximeter, Pursed Lip Breathing, Tracheostomy - Deep Breathing and Coughing, Tracheostomy Suctioning, Tracheostomy Tubes, Tracheostomy-Laryngectomy - When to Call Hospice, Tuberculosis, Using a Spacer with Your Inhaler, When You're Feeling Short of Breath	11/20/2025 1:57 AM	TRACY KING RN

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
3	potential for Constipation	2	If patient reports improved symptoms with use of oxygen, continue oxygen therapy. Instruct patient and family/caregiver on relaxation techniques:, Guided imagery, Therapeutic touch, Music Provide Patient Education Handout(s):, Food Sources of Magnesium, Low Blood Magnesium, Paracentesis, Removal of a Breathing Machine, Anemia-Related Fatigue, About Your Oxygen, BiPAP and CPAP, Breathing Exercises, CPAP, Dry Powder Inhaler (DPI), How to Count Pulse and Respirations, Humidifiers and Vaporizers, Laryngectomy Suctioning, Metered Dose Inhaler (MDI), Nebulizer Treatments, Oxygen - Cleaning and Maintaining System, Oxygen Concentrator, Oxygen Concentrator - Manufacturer's Operating Instructions, Oxygen Safety, Pulse Oximeter, Pursed Lip Breathing, Tracheostomy - Deep Breathing and Coughing, Tracheostomy Suctioning, Tracheostomy Tubes, Tracheostomy-Laryngectomy - When to Call Hospice, Tuberculosis, Using a Spacer with Your Inhaler, When You're Feeling Short of Breath	12/3/2025 10:53 PM	TRACY KING RN
4	potential for Constipation	2	If patient reports improved symptoms with use of oxygen, continue oxygen therapy. Instruct patient and family/caregiver on relaxation techniques:, Guided imagery, Therapeutic touch, Music Provide Patient Education Handout(s):, Food Sources of Magnesium, Low Blood Magnesium, Paracentesis, Removal of a Breathing Machine, Anemia-Related Fatigue, About Your Oxygen, BiPAP and CPAP, Breathing Exercises, CPAP, Dry Powder Inhaler (DPI), How to Count Pulse and Respirations, Humidifiers and Vaporizers, Laryngectomy Suctioning, Metered Dose Inhaler (MDI), Nebulizer Treatments, Oxygen - Cleaning and Maintaining System, Oxygen Concentrator, Oxygen Concentrator - Manufacturer's Operating Instructions, Oxygen Safety, Pulse Oximeter, Pursed Lip Breathing, Tracheostomy - Deep Breathing and Coughing, Tracheostomy Suctioning, Tracheostomy Tubes, Tracheostomy-Laryngectomy - When to Call Hospice, Tuberculosis, Using a Spacer with Your Inhaler, When You're Feeling Short of Breath	12/3/2025 10:54 PM	TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

CARE PLAN

Problem being care planned: Edema/Lymphedema
Measurable Goals: Patient and family/caregiver will report understanding of management of edema.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for lymphedema:, Skin care, External compression or support, Manual lymph drainage, Exercise	11/7/2025 3:58 AM	TRACY KING RN
2	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for lymphedema:, Skin care, External compression or support, Manual lymph drainage, Exercise	11/20/2025 1:56 AM	TRACY KING RN
3	Edema/Lymphedema	1	Assess for possible causes of dependent edema: Treatment with artificial fluids, steroids, estrogens, Deep vein thrombosis, Immobility, End-stage cardiac disease, Other chronic illness Instruct patient and family/caregiver on treatment for lymphedema:, Skin care, External compression or support, Manual lymph drainage, Exercise	12/3/2025 10:52 PM	TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

CARE PLAN

Problem being care planned: Nausea/Vomiting

Measurable Goals: Family/caregivers will verbalize understanding of the management of nausea and vomiting.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family on altering meal patterns to have patient eat when least nauseated.	11/7/2025 3:59 AM	TRACY KING RN
2	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family on altering meal patterns to have patient eat when least nauseated	11/20/2025 1:55 AM	TRACY KING RN
3	Nausea/Vomiting	1	Assess potential causes of nausea and vomiting: Medication, Initiation of opioids, Infectious process, Brain metastases, Anxiety, Ascites, Gastric irritation, Constipation, Hypercalcemia, Intestinal obstruction, Uremia, Unrelieved pain, Vertigo Instruct patient/family on altering meal patterns to have patient eat when least nauseated	12/3/2025 10:51 PM	TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

CARE PLAN

Problem being care planned: Pain

Measurable Goals: Patient will report that pain is reduced to the level personally desired: _____ on a scale of 1-10

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	potenial for pain	1	Instruct patient and family on the difference between tolerance and addiction and that increasing doses of pain medication are expected. Assess patient and family concerns related to the use of narcotic pain medications.	11/7/2025 4:00 AM	TRACY KING RN
2	potenial for pain	1	Instruct patient and family on the difference between tolerance and addiction and that increasing doses of pain medication are expected. Assess patient and family concerns related to the use of narcotic pain medications.	11/20/2025 1:54 AM	TRACY KING RN
3	potenial for pain	1	Instruct patient and family on the difference between tolerance and addiction and that increasing doses of pain medication are expected. Assess patient and family concerns related to the use of narcotic pain medications.	12/3/2025 10:50 PM	TRACY KING RN

Patient: SANDRA HAMLIN

Medical Record: #90008

CARE PLAN

Problem being care planned: Diarrhea

Measurable Goals: Patient and family will report understanding of disease process and management of loose, frequent stools.

Update #	Problem description	Severity score	Interventions/outcomes	Date	Signature
1	potential for Diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautions	11/7/2025 4:02 AM	TRACY KING RN
2	potential for Diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautions	11/20/2025 1:53 AM	TRACY KING RN
3	potential for Diarrhea	1	Assess for cause of diarrhea: Misuse of laxatives, Fecal impaction, Tumor involvement of intestinal tract, Medications, Infectious process, AIDS disease process , Tube feedings, Lactose intolerance, Sorbitol used in elixirs and enteral feedings, Anxiety , Other Provide patient education handout(s): Avoiding Gastric Stimulants (Ulcer Diet), Diet for Controlling Abdominal Gas, Diet for Diverticulosis, Diet for Gastroparesis, High Fiber Diet, Malabsorption Diet, Nutrition, Concerns at the End of Life, Anal Fissures, Managing Diarrhea, Clostridium Difficile, Hand Hygiene, Hand Washing for Caregivers, Handwashing, Preventing the Spread of Infection, VRE Contact Precautio	12/3/2025 10:49 PM	TRACY KING RN

Patient: SANDRA HAMLIN
Medical Record: #90008

IDG UPDATE

From date: 11/20/2025
To date: 12/4/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update: All equipment was provided at admission

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
No medications									
Comfort Kit Medications:									
11/4/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/4/2025		Atropine sulfate oral Solution	0.75%	1-2 drops	Take Sublingually	every 2 hours as needed	for excess secretions - #15mL / RF: 11	Yes	Active
11/4/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF:11	Yes	Active

11/4/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/4/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/4/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/4/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/4/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF:11	Yes	Active

Medication list reflects medication orders as of 11/20/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	11/7/2025	11/20/2025
Constipation	11/7/2025	11/20/2025
Dyspnea	11/7/2025	11/20/2025
Edema/Lymphedema	11/7/2025	11/20/2025
Nausea/Vomiting	11/7/2025	11/20/2025
Pain	11/7/2025	11/20/2025
Diarrhea	11/7/2025	11/20/2025

Care Plan list reflects care plan orders as of 11/20/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: Cerebral Infarction, unspecified. Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	11/4/2025	
TREATMENT ORDERS - Trazodone 150mg. Take 1 tablet at bedtime for insomnia Tylenol 500mg . Take 1 tablet by mouth q6hrs prn for pain	11/20/2025	

Physician orders list reflects physician orders as of 11/20/2025

IDG Narrative: The pt still has edema in the right arm and takes furosemide to manage it. The pt's caregiver said that the Trazodone is not helping the pt with getting sleep at night. Sn consulted Dr. Restrepo who prescribed Trazodone 150mg at bedtime. SN called in the medication. The pt is also sleepier during the day and the caregiver said that she tries to keep the pt busy and engaged during the day so that she can rest/sleep better at night.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: JAMES KIZITO RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor RICK WATTS M.DIV
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 11/20/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: RICK WATTS M.DIV
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

IDG UPDATE

From date: 12/4/2025
To date: 12/18/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update: All equipment was provided at admission

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
12/3/2025		Furosemide	20	1	By mouth	Twice a day	Edema	Yes	Active
12/3/2025		Melatonin	10	1	By mouth	Qhs	Insomnia	Yes	Active
12/3/2025		Potassium	10	1	Po	Daily	Supplement	Yes	Active
12/3/2025		Quetiapine	50	1	Po	At bedtime	Mood disorder	Yes	Active
Comfort Kit Medications:									
11/4/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/4/2025		Atropine sulfate oral	0.75%	1-2 drops	Take Sublingually	every 2 hours as	for excess secretions -	Yes	Active

		Solution				needed	#15mL / RF: 11		
11/4/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF: 11	Yes	Active
11/4/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/4/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/4/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/4/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/4/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF: 11	Yes	Active

Medication list reflects medication orders as of 12/4/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	11/7/2025	12/3/2025
Constipation	11/7/2025	12/3/2025
Dyspnea	11/7/2025	12/3/2025
Edema/Lymphedema	11/7/2025	12/3/2025
Nausea/Vomiting	11/7/2025	12/3/2025
Pain	11/7/2025	12/3/2025
Diarrhea	11/7/2025	12/3/2025

Care Plan list reflects care plan orders as of 12/4/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: Cerebral Infarction, unspecified. Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	11/4/2025	
TREATMENT ORDERS - Trazodone 150mg. Take 1 tablet at bedtime for insomnia Tylenol 500mg . Take 1 tablet by mouth q6hrs prn for pain	11/20/2025	

Physician orders list reflects physician orders as of 12/4/2025

IDG Narrative:

Pt continues to have 3+ edema to her bil lower extremities.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

- Medical Director/Physician Designee: DIEGO F RESTREPO MD
- Registered Nurse: STELLA AHANOTU RN
- Social Worker: ASHLEY DAVIS LMSW
- Spiritual Counselor: CLARK MAYS
- Volunteer Coordinator: VERNON MORRISON
- Bereavement Coordinator: VERNON MORRISON

Date: 12/4/2025

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: VERNON MORRISON
Electronically signed by: CLARK MAYS
Electronically signed by: DIEGO F RESTREPO MD

Patient: SANDRA HAMLIN
Medical Record: #90008

IDG UPDATE

From date: 12/4/2025
To date: 12/18/2025

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits 0

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

☒ All the patient’s medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

IDG Narrative: Pt continues to have 3+ edema to her bil lower extremities.

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: STELLA AHANOTU RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor: CLARK MAYS
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 12/4/2025

Electronically signed by: VERNON MORRISON
Electronically signed by: DIEGO F RESTREPO MD

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: CLARK MAYS

Patient: SANDRA HAMLIN
Medical Record: #90008

IDG UPDATE

From date: 12/18/2025
To date: 1/1/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptoms management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

IDG Narrative: Continue with poc

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

Medical Director/Physician Designee: DIEGO F RESTREPO MD
Registered Nurse: STELLA AHANOTU RN
Social Worker: ASHLEY DAVIS LMSW
Spiritual Counselor: CLARK MAYS
Volunteer Coordinator: VERNON MORRISON
Bereavement Coordinator: VERNON MORRISON

Date: 12/16/2025

Electronically signed by: DIEGO F RESTREPO MD
Electronically signed by: STELLA AHANOTU RN

Electronically signed by: VERNON MORRISON
Electronically signed by: CLARK MAYS

Patient: SANDRA HAMLIN
Medical Record: #90008

IDG UPDATE

From date: 12/18/2025
To date: 1/1/2026

Planned Visit Frequencies - Plus PRN Visits for Symptom Management

Nursing

Weekly Visit Frequency: 1
+PRN Weekly Visits 1
Reason for PRN visits Symptom Management

Social Worker

Visit Frequency in next month: 1
+PRN Visits in next month 1

Spiritual

Visit Frequency in next month: 1
+PRN Visits in next month 1

Hospice Aide

Weekly Visit Frequency: 5

Updates to Durable Medical Equipment

Describe any changes to Durable Medical Equipment pertaining to the patient since the last IDG Update: All equipment was provided at admission

Patient/Family Involvement in Reviewing and Revising Plan of Care

☒ Updates to Plan of Care were discussed with the patient/family member who verbalizes understanding and agreement with Plan of Care

Attending Physician Involvement in Reviewing and Revising Plan of Care

Comments from Attending Physician: The hospice doctor is the attending physician.
☒ Attending Physician consulted with and verbalizes agreement with Plan of Care:

Plan of Care Review

Current Medications:

Start Date	DC Date	Medication	Unit Strength	Dose	Route	Frequency	Indication	Covered By Hospice	Status
Regular Medications:									
12/3/2025		Furosemide	20	1	By mouth	Twice a day	Edema	Yes	Active
12/3/2025		Melatonin	10	1	By mouth	Qhs	Insomnia	Yes	Active
12/3/2025		Potassium	10	1	Po	Daily	Supplement	Yes	Active
12/3/2025		Quetiapine	50	1	Po	At bedtime	Mood disorder	Yes	Active
Comfort Kit Medications:									
11/4/2025		APAP 650MG SUPPS	650 mg	1 suppository	unwrap and insert Rectally	Q 4 HRS PRN	Pain/Fever - #4 / RF:11	Yes	Active
11/4/2025		Atropine sulfate oral	0.75%	1-2 drops	Take Sublingually	every 2 hours as	for excess secretions -	Yes	Active

		Solution				needed	#15mL / RF: 11		
11/4/2025		Bisacodyl suppository	10 mg	1 suppository	unwrap and insert Rectally	Q DAY PRN	Constipation - #3 / RF: 11	Yes	Active
11/4/2025		Lorazepam Liquid	1.75mg/ml	0.5-1 mg	PO/SL	q4 hrs prn	Anxiety/SOB - #30mL / RF: 5	Yes	Active
11/4/2025		Morphine Sulfate	100mg/5ml = 20 mg/ml	0.25-1ml	Sublingual	Q1-2 hours prn	Pain/SOB - #120mL	Yes	Active
11/4/2025		Oxygen	na	2-4 liters	intranasally	PRN	Shortness of breath	Yes	Active
11/4/2025		Senna	8.6mg	1 Tab	PO	Daily	Constipation	Yes	Active
11/4/2025		Zofran ODT	1 tablet	4mg	dissolve PO/SL	Q 8 HRS PRN	nausea/vomiting - #6 / RF: 11	Yes	Active

Medication list reflects medication orders as of 12/18/2025

☒ All the patient's medications have been reviewed, including but not limited to identification of the following: effectiveness of drug therapy, drug side effects, actual or potential drug interactions, duplicate drug therapy, drug therapy currently associated with laboratory monitoring.

Current Care Plans:

Problem	Start Date	Latest Update
Anxiety/Agitation	11/7/2025	12/3/2025
Constipation	11/7/2025	12/3/2025
Dyspnea	11/7/2025	12/3/2025
Edema/Lymphedema	11/7/2025	12/3/2025
Nausea/Vomiting	11/7/2025	12/3/2025
Pain	11/7/2025	12/3/2025
Diarrhea	11/7/2025	12/3/2025

Care Plan list reflects care plan orders as of 12/18/2025

Treatment Order List:

Name	Start Date	DC Date
TREATMENT ORDERS - Admit to Hospice: Dx: Cerebral Infarction, unspecified. Patient is an OOH-DNR Diet & Activity as tolerated DME: Hospital Bed, Bedside Table, BSC, Wheelchair	11/4/2025	
TREATMENT ORDERS - Trazodone 150mg. Take 1 tablet at bedtime for insomnia Tylenol 500mg . Take 1 tablet by mouth q6hrs prn for pain	11/20/2025	

Physician orders list reflects physician orders as of 12/18/2025

IDG Narrative:

Continue with poc

As the Interdisciplinary Team members, we have participated in reviewing and planning this patient's care:

- Medical Director/Physician Designee: DIEGO F RESTREPO MD
- Registered Nurse: STELLA AHANOTU RN
- Social Worker: ASHLEY DAVIS LMSW
- Spiritual Counselor: CLARK MAYS
- Volunteer Coordinator: VERNON MORRISON
- Bereavement Coordinator: VERNON MORRISON

Date: 12/18/2025

Electronically signed by: STELLA AHANOTU RN
Electronically signed by: DIEGO F RESTREPO MD
Electronically signed by: VIRGINIA ALLAN RN ADON
Electronically signed by: VERNON MORRISON
Electronically signed by: CLARK MAYS

Patient: SANDRA HAMLIN
Medical Record: #90008

SPIRITUAL PROGRESS NOTE

SPIRITUAL PROGRESS NOTE

Date/Time in: 11/13/2025 1:15:00 PM
Date/Time out: 11/13/2025 1:31:00 PM
Purpose of progress note: Telehealth Visit

Narrative: Chaplain initiated a follow-up call to the daughter of patient Sandra Hamlin following the patient's recent hospital stay and readmission to hospice services. The daughter reported that the patient's condition has stabilized, with no significant changes since returning home. She expressed appreciation for the Chaplain's outreach and stated that the family continues to feel well supported by the care team.

Date: 11/13/2025

Electronically signed by: DAMIEN DAWSON M.DIV